SAGASIA

SEA Healthcare Sector Outlook

September 2026

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Initial Findings

Healthcare Trends and Themes Across SEA

Healthcare across Southeast Asia is shifting toward a more digital, preventive, and patient-centric model, with strong growth in private healthcare, outpatient services, and diagnostics.

Key Trends



Expansion of telemedicine and digital platforms

Strongest adoption in **Singapore** and **Malaysia**



Shift toward alternative healthcare settings

Movement away from **large hospitals toward home care** and **day surgery centres**



Acceleration of private-sector expansion

Fuelled by the rising **middle class**, **long public wait times**, and **medical tourism**



Growing importance of preventive care and diagnostics

Diagnostics, **screening**, and **early intervention** are growing segments

Key Themes



Digital health and telemedicine adoption



Preventive care and diagnostics growth



Private hospital expansion

Shift toward outpatient care

Sources: NST Online, the Sun, King's College London, MOHSingapore, Japan Valuers, ARC Group & Asian Insiders

Technology Is Enabling New Care Models Across SEA

Following country-specific demand drivers, healthcare providers across Southeast Asia are using digital tools and integrated delivery models to expand access, improve efficiency, and create new growth models.



AI-enabled diagnosis and decision support

Supports faster image analysis, triage, and clinical decision-making, **improving productivity** and **diagnostic consistency**



Automation of healthcare operations

Streamlines **administrative workflows**, **laboratory processes**, **pharmacy fulfilment**, and **documentation**



Integrated health data and electronic medical records

Connects patient information across **care settings**, **enabling more coordinated** and **continuous care**



Remote patient monitoring and connected devices

Wearables and connected devices allow **continuous monitoring chronic conditions** such as diabetes, hypertension and heart disease



Expansion of telemedicine and virtual care

Patients can access **consultations**, **follow-ups**, **prescriptions** through digital platforms. Telehealth has improved access to care **especially in rural areas**

New Business Models Emerging



Integrated healthcare ecosystem



Asset-light expansion



Hybrid care (Online & Offline)



Value-based care

Sources: Deloitte, Southeast Asia Development Solutions & NUS

Country-Specific Healthcare Opportunities Across SEA

Healthcare opportunities across the region are increasingly differentiated, with each market shaped by medical tourism, ageing care, digital adoption, or capacity expansion.



Malaysia

- Established healthcare-travel hub supported by **strong private hospital groups** and **transparent treatment costs**
- **High chronic-disease** burden supports demand for **diagnostics**, **screening**, and **long-term disease management**



Singapore

- Regional referral centre for premium and complex care, backed by **advanced clinical infrastructure**
- Fast-ageing population supports **eldercare, home care**, and **AI-enabled care coordination**



Thailand

- Leading medical-tourism market anchored by **well-known private hospital networks**
- Ageing society is accelerating demand for **rehabilitation**, **outpatient care**, and **senior-care services**



Indonesia

- Large underserved archipelago requires **continued hospital**, **clinic**, and **diagnostic expansion**
- **National health insurance** and **digital health** are improving healthcare access and utilization



Vietnam

- **Rising healthcare expenditure** and **persistent household spending** are supporting demand for accessible, higher-quality services
- Hospital upgrades are driving investment in **equipment**, **diagnostics**, and **specialty services**

Sources: Malaysia Healthcare Travel Council, WHO, Singapore Ministry of Health, Singapore EDB, Thailand BOI, UNFPA, World Bank and Reuters

Stakeholders & Core Demand Drivers

Patients	Higher healthcare awareness and demand for faster access
Insurers	Shifts spending from out of pocket to insured care
Corporates	Employer-sponsored insurance and corporate wellness, especially in Singapore, Malaysia and Thailand
Government buyer	Hospital infrastructure investment and broader healthcare access

Demographics

- Aging population increasing healthcare utilization
- Longer life expectancy is driving chronic-care demand

Income & Middle-Class Growth

- Rising disposable income is boosting private healthcare spending
- Strong demand for premium healthcare services

Urbanization

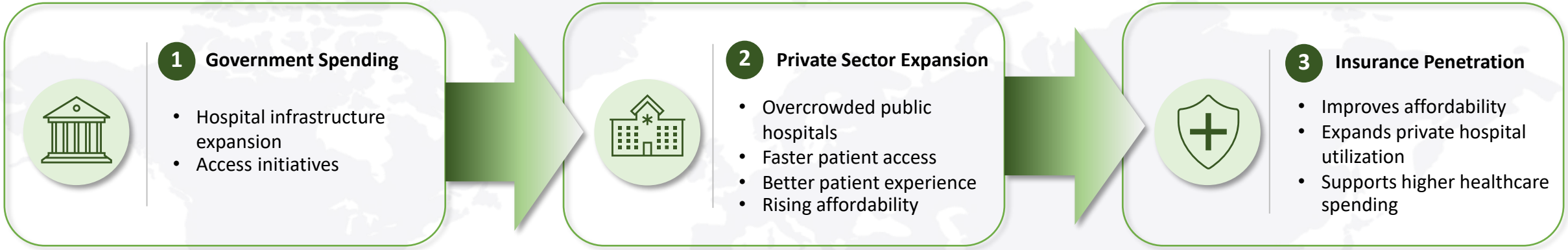
- Higher healthcare accessibility in urban centres
- Growth of private hospitals and specialty clinics

Chronic Diseases / NCDs

- Rising diabetes, cancer, cardiovascular diseases
- Drives recurring diagnostics and long-term treatment demand

Sources: MIDF research, MOHSingapore, MoPH & Dezan Shira & Associates

Healthcare Ecosystem Growth Drivers



Medical Tourism Positioning

Country	Positioning	Key Medical Tourism Driver
Malaysia	Regional medical hub	Middle East inflows + cost competitiveness
Singapore	Premium healthcare destination	Indonesian high-income patients
Thailand	Value-for-money healthcare	Large Indonesian patient inflows

Source: US-ASEAN Business Council

Healthcare Regulatory Framework

Regulatory frameworks across SEA are evolving to balance healthcare expansion with stronger compliance, patient safety, and digital health oversight.

Country	Regulatory Body	Regulations
Malaysia 	• Ministry of Health Malaysia • National Pharmaceutical Regulatory Agency • Malaysian Medical Council	• Private Healthcare Facilities and Services Act 1998 • Medical Act 1971 • Poisons Act 1952 / Sale of Drugs Act 1952 –Telemedicine Act 1997 (not fully enforced but relevant)
Singapore 	• Ministry of Health Singapore • Health Sciences Authority • Singapore Medical Council	• Healthcare Services Act • Medicines Act • Personal Data Protection Act (PDPA)
Thailand 	• Ministry of Public Health Thailand • Thai Food and Drug Administration	• Private Hospital Act • Medical Profession Act • Drug Act
Indonesia 	• Ministry of Health Indonesia • BPOM (Food and Drug Authority)	• Health Law No. 17 of 2023 • Hospital Law no. 44/2009 • Medical Practice Law No. 29/2004
Vietnam 	• Ministry of Health Vietnam • Drug Administration of Vietnam	• Law on medical examination and treatment • Law on pharmacy • Law on health insurance

Source: MOHM, MOHSingapore, MoPH, Indonesian MoH & Vietnam's MoH

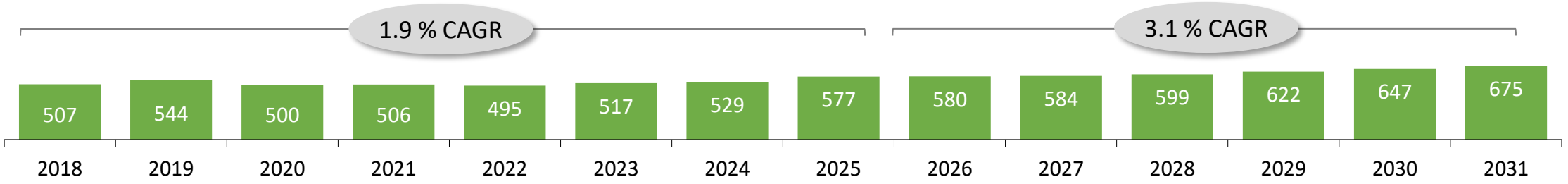
Macroeconomics

Thailand's GDP is projected to expand at a CAGR of 3.1% over 2026–2031, supported by recovering domestic demand, increased investment activity, and ongoing structural improvements to the business landscape.



Socio-economic Statistics	
GDP (2025)	USD 577 Bn
GDP / Capita (2025)	USD 8,057
GDP Growth (2026-2032)	3.1% CAGR
Population (2025)	71.56 Mn
Urbanisation (2025)	54.3%
Inflation rate (2025)	0.3%

Thailand GDP – Historical and Forecasts (USD Bn)



- **Thailand's GDP growth moderated to 2.4% in 2025**, with improving consumption and investment activity indicating a gradual stabilization in underlying demand conditions.
- **The recovery in construction and increased public spending** are supporting industrial activity, contributing to a more favorable operating environment across key sectors.

Sources: International Monetary Fund (IMF), Krungsri Research Economic Outlook 2026, Office of the National Economic & Social Development Council

Demographic & Culture

Thailand's 2026 market is a resilient, high-value digital hub defined by a "Polarized Market" of high connectivity and "sanuk" (joyful) culture. As Southeast Asia's second-largest digital economy, Thailand offers a large consumer base supported by ongoing digital trends.



1 Culture

Thai behavior is driven by **the pursuit of joy**; winning brands create emotional resonance and shareable moments.

2 Wellness Infrastructure

Healthcare economy supported by heavy state funding for **digitized provincial health centers** and **local medical supply chains**.

3 Consumer Trends

Highly tech-dependent consumers expect **seamless mobile payments** and **localized app ecosystems** across digital commerce.

4 Healthcare Expansion

State-backed Universal Coverage Scheme (UCS) extends digital therapeutics and remote-clinic access to **over 99% of citizens**.

5 Smart Value

Amid high household debt, consumers seek "**smart value**" aspirational lifestyle with practical, cost-effective choices.

6 Global Medical Hub

Thailand leads wellness travel, generating **$4B+ annually** through affordable surgeries and luxury recovery.

Source: Kadence, NHSO, Digital in Asia, PR Dept, Krungsri, Thailand.go.th

Total Household Spending

Thailand's robust household spending, led by Bangkok, reveals clear premium and mid-tier segments, highlighting strong, scalable demand for market entry and expansion.

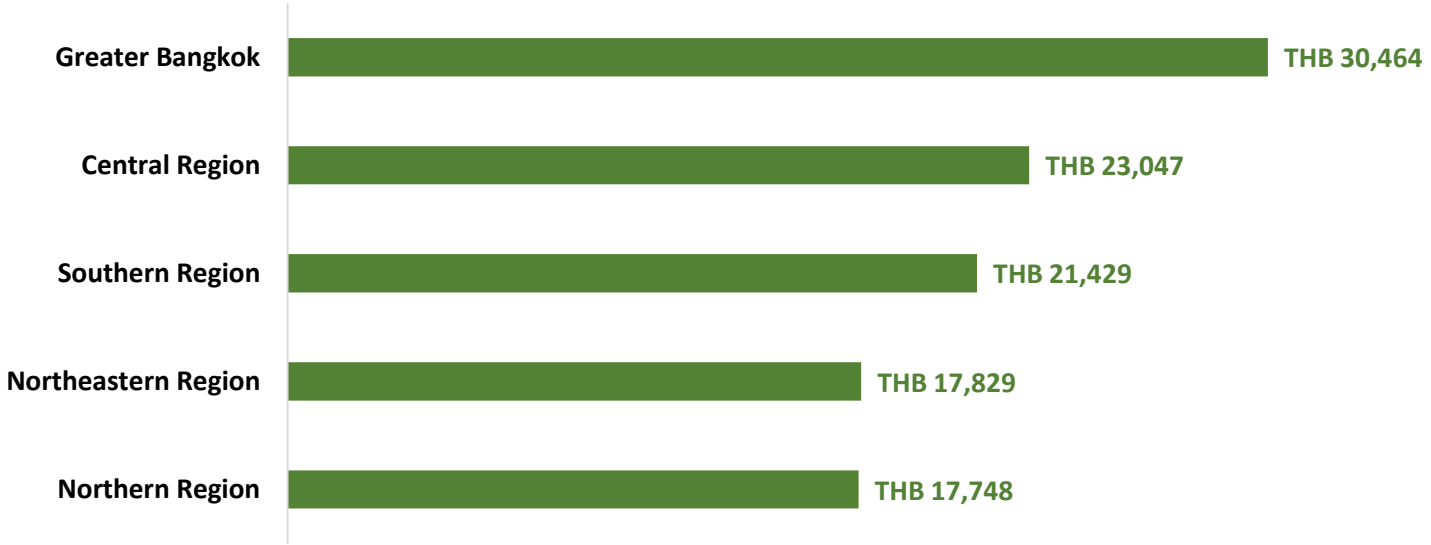
National Monthly Household Expenditure

22,282 B

CAGR Growth of Household Expenditure

3.3%

Regional Spending Comparisons in THB

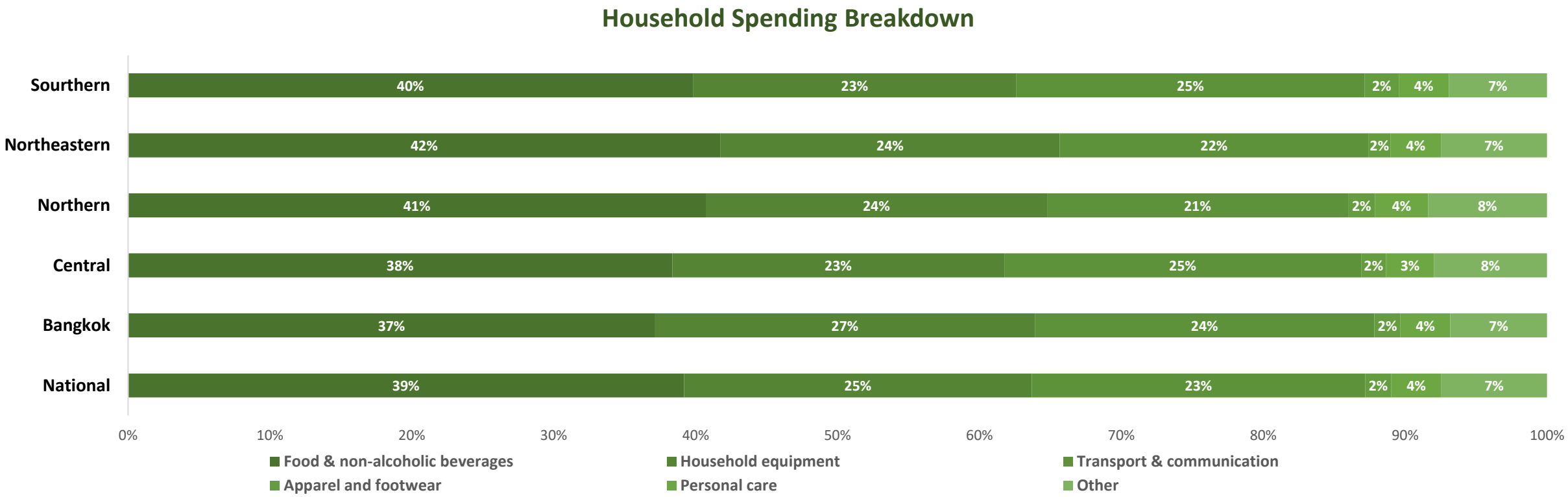


- Spending in **Greater Bangkok (฿30,464)** is roughly **37% higher** than the national average and nearly **70% higher** than the North/Northeastern regions.
- The Central and Southern regions represent a strong "Middle Tier," with spending levels significantly above the 20k mark.

Sources: Statistical Yearbook Thailand 2025

Total Household Spending Breakdown

Thai household spending is highly concentrated in essentials, with limited discretionary spend, indicating value-driven consumers and strong demand for affordable, functional products.



- Thai households allocate **nearly 90%** of their total budget to just three categories: **Food & Beverages (39.0%), Household Equipment (25%), and Transport & Communication (23%)**
- The average monthly spend on **Personal Care (679 Baht) and Apparel & Footwear (357 Baht) is relatively low** compared to essentials.
- Thai households **spend significantly on Transport and Communication (4,539 Baht),** which is nearly 7x what they spend on personal care.

Sources: Statistical Yearbook Thailand 2025

Healthcare Value Chain

Thailand combines broad public access with strong private value pools, constrained by workforce and cost pressures.

1 Policy & regulation

(Thai FDA)

THAILAND BOARD OF INVESTMENT

NHSO National Health Security Office

(SSO)

(Medical Council)

2 Payers

สำนักงานประกันสังคม SOCIAL SECURITY OFFICE

UCS

- Thailand's Civil Servant Medical Benefit Scheme (CSMBS)
- private insurance
- self-pay
- foreign patients

3 Providers

Bumrungrad International HOSPITAL

BDMS Bangkok Dusit Medical Services

CHG จุฬารัตน์

โรงพยาบาลกรุงเทพคริสเตียน THE BANGKOK CHRISTIAN HOSPITAL

Praram 9 Hospital

4 Inputs

GPO Government Pharmaceutical Organization

- Local/MNC pharma
- Medtech suppliers
- Diagnostics
- Hospital IT

5 Access / customers

- Public patients
- SSO workers
- Civil servants
- Domestic private
- Medical/wellness tourists

Where commercial value is captured

Premium hospitals

BDMS/BH capture high-margin domestic, insured, and foreign patient demand

Medical tourism & wellness

Care bundled with hospitality, check-ups, rehab, longevity, and recovery

SSO-linked provider

BCH/CHG capture domestic worker-volume demand but need tight cost discipline

Diagnostics / pharma / medtech

Recurring NCD and aging demand supports longitudinal value pools

Main value-chain bottlenecks

Workforce competition

between public and private hospitals

Aging and chronic disease pressure

on UCS/SSO budgets

Foreign-patient volatility

from travel, geopolitics, FX, and regional competition

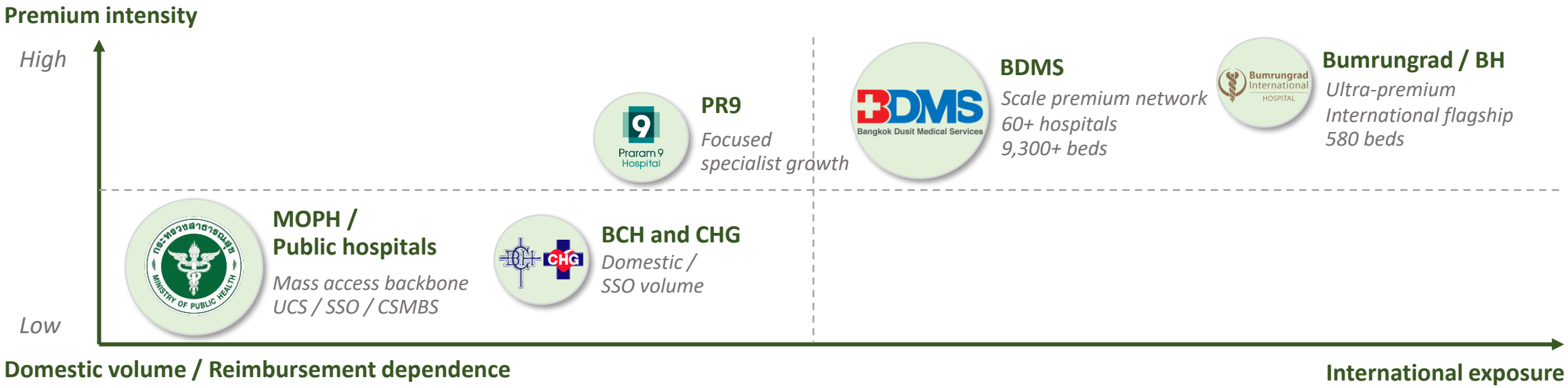
High-end device and specialty-drug

import dependence

Sources: Sagasia internal research based on Thailand BOI Healthcare and Wellness Industry, MOPH FY2025 Action Plan, TDRI Universal Healthcare analysis, and Krungsri Research healthcare industry outlooks.

Providers Landscape

Thailand's provider market is divided between volume-driven public care and premium, internationally exposed private players.



Market structure

MOPH / Public hospitals
- Affordability
- Access
- Public health
- Emergency / referral care

BDMS
- Premium multi-brand network spanning domestic
- foreign demand

Bumrungrad / BH
- High-acuity foreign patient brand

BCH and CHG
- Domestic middle-market
- SSO-linked volume

PR9
- Upper-mid / premium specialist platform

Strategic reading

- **Premium** and **international care** drive the strongest value differentiation
- **SSO-linked players** compete on **volume** and **cost discipline**
- **Network scale** matters most in **premium** and **multi-city coverage**
- **Reimbursement dependence** remains a key dividing line across providers

Sources: Sagasia internal research based on Finansia Syrus Thailand Healthcare report, BDMS Investor Relations, Bumrungrad Investor Relations, and Thanachart Securities Healthcare Sector Report.

Fixed

Customer Segments & Demand-Side View

Thailand has broad healthcare demand, covering treatment, prevention, recovery, wellness and elderly care across both public and private patients.

Key Customer Segments & Demand

Mass Public Patients
Affordable essential care, chronic disease treatment, public access

Formal-sector workers
SSO-covered care, convenient contracted hospitals

Domestic private patients
Speed, comfort, specialists, diagnostics, private rooms

Foreign medical tourists
Trust, accreditation, packages, language/concierge support

Wellness / preventive users
Longevity, check-ups, rehab, anti-aging, medical wellness

Elderly / NCD patients
Recurring care, medicines, diagnostics, rehab, monitoring

Likely Provider Choice

Mass Public Patients	MOPH hospitals, UCS-contracted providers
Formal-sector workers	SSO hospitals; BCH/CHG-type providers
Domestic private patients	BDMS, PR9, Bumrungrad, private hospitals
Foreign medical tourists	Bumrungrad, BDMS brands, specialist hospitals
Wellness / preventive users	Wellness clinics, private hospitals, resorts
Elderly / NCD patients	Public/private hospitals, clinics, pharmacies

Quantitative demand signals

- **Broad access:** UHC index 82, with OOP at 8.8% of CHE
- **Strong international trust:** 65 JCI accredited healthcare facilities
- **Demand growth:** Aging, NCDs, insurance and foreign patient flows

Provider implications

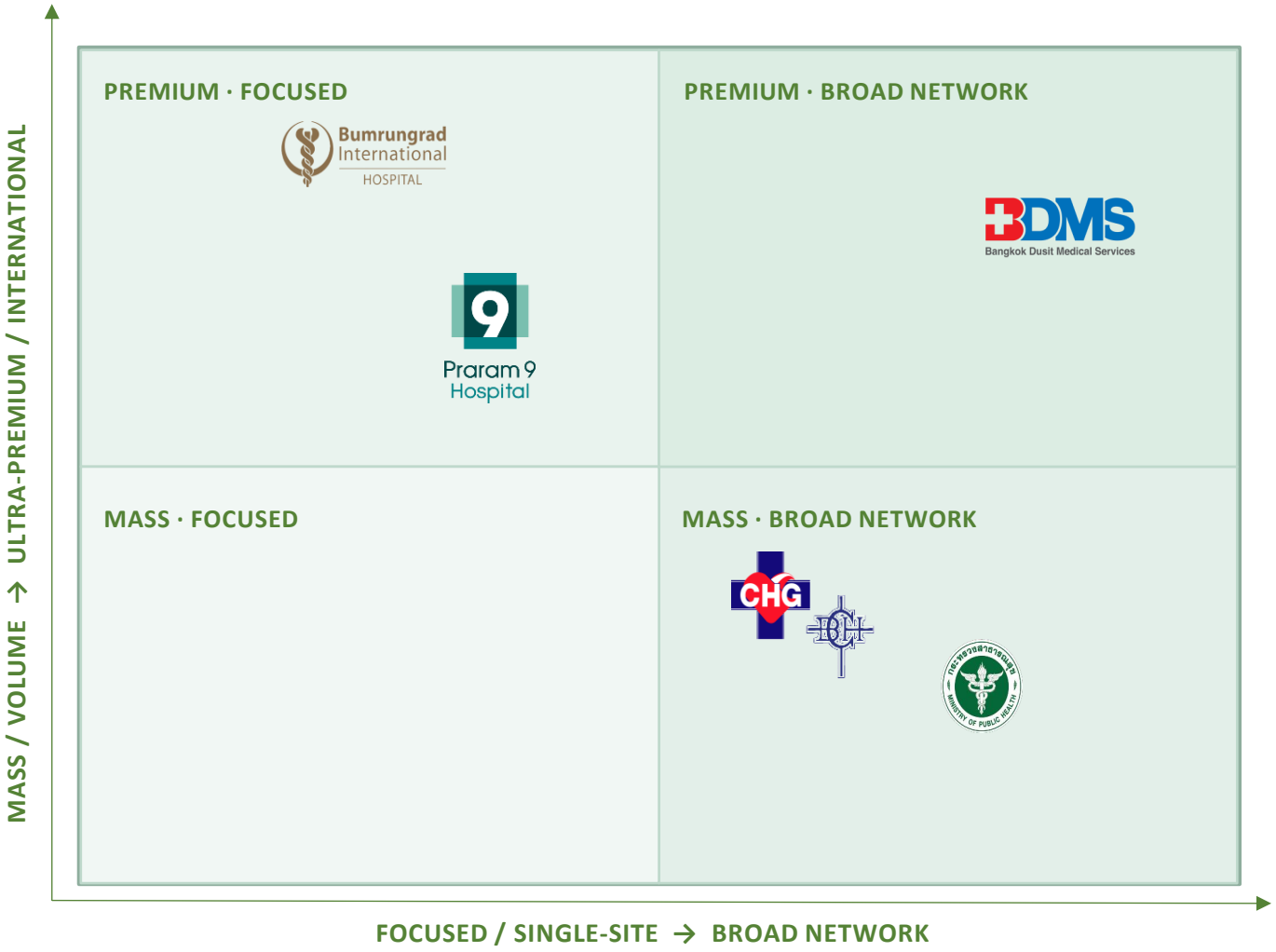
- **Differentiate premium care:** International services, specialists and wellness bundles
- **Protect SSO margins:** Tight volume management and cost control
- **Target aging demand:** Diagnostics, chronic care, medtech and rehabilitation

Sources: Sagasia internal research based on WHO Thailand SDG Profile 2025, Thailand BOI Healthcare and Wellness Industry, Finansia Syrus Thailand Healthcare report, and TDRI Universal Healthcare analysis.

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Brand Positioning Axis

Thailand's healthcare winners compete on scale or specialization. High entry barriers mean no player has built a low-cost, focused model.



Why disruption is difficult

- Capital & Licensing barriers
- Scarce specialist talent
- Trust & brand drive patient choice
- Complex reimbursement

How leaders defend their position

- **BDMS**: Scale & referral network
- **Bumrungrad**: Global premium brand
- **BCH/CHG**: SSO-driven volume model
- **Public hospitals**: Affordability and access

What drives success

- Clear positioning
- Specialist depth = differentiation
- International play = trust & patient experience
- Scale = cost & throughput control

*Sources: Sagasia internal research; BDMS/Bumrungrad/BCH/CHG/PR9 FY2024 financials; Thanachart Securities Healthcare Sector Report; Thailand FY2024 budget (Ministry of Public Health). *Public bubble = FY2024 MOPH budget (THB165.7bn), not revenue — shown for scale reference only, not directly comparable to the private figures. Quadrant position reflects number of operating sites (BCH: 16 hospitals; CHG: 14 branches), not revenue.*

Political Outlook

Thailand's healthcare sector remains a government priority, though political transition may delay near-term funding and policy rollout. Short-term uncertainty may slow execution, but structural policy backing for healthcare remains strong.

Universal Coverage Scheme

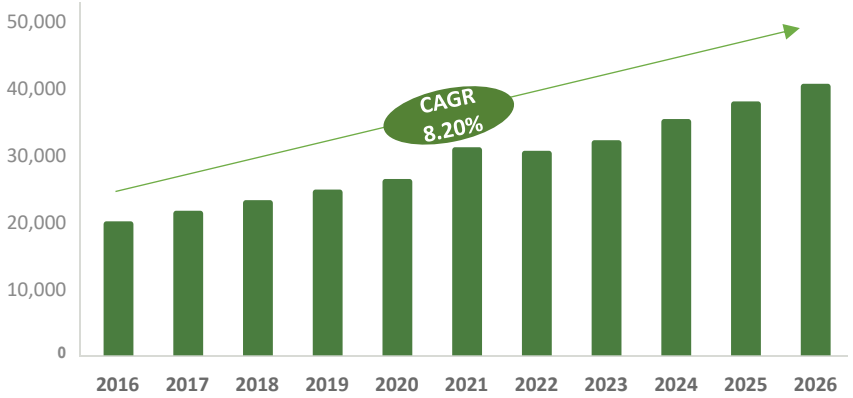
47 Mn
citizens covered

USD 7.9 Bn
annual funding

Financing the backbone of Thailand's healthcare system

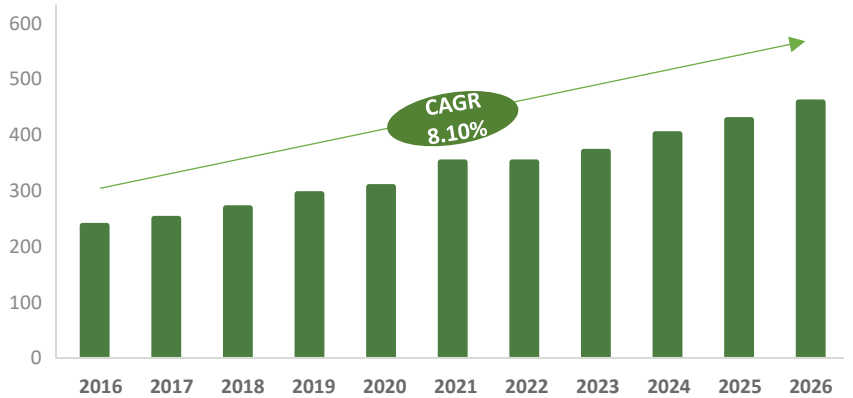
Healthcare spending growing ~8% CAGR

Expenditure on healthcare in Thailand (USD)



Per capita spend rising steadily

Spending per capita on healthcare in Thailand (USD)



Short-Term Disruptions

- Political transition → delays
- Budget disbursement slows
- Approvals lag to 2026

Macro Spillover

- GDP slowdown (1.8–2.0%)
- External pressure (tariffs)
- Private sector cautious

Policy Direction

Healthcare = strategic sector
Key priority under Medical Hub strategy

Strong public funding (~70%)
Spending projected to exceed THB 1.08T by 2027 (~70% public)

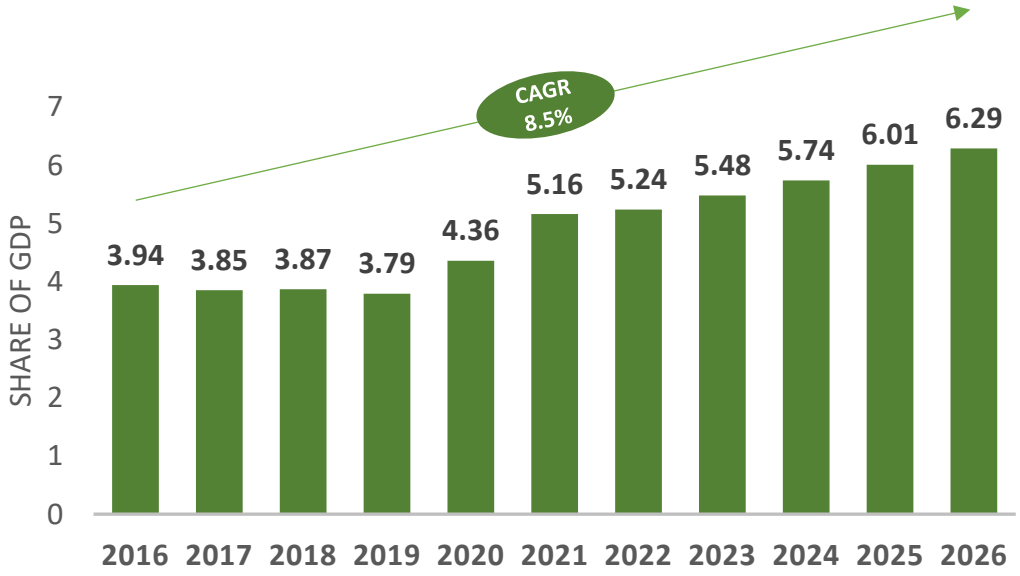
Sources: Statista, Krungsri, KAsset, NHSO

Saw – in progress

Economic Outlook

Healthcare in Thailand is expanding faster than the broader economy, driven by structural demand from aging and medical hub positioning. At the same time, rising medical costs are creating affordability pressures that are reshaping patient behavior and access.

GDP share of Health Expenditure in Thailand from 2016-2026



Growth Drivers

- Healthcare spending growing at **~8.5% CAGR (2026–2032)** vs GDP at ~2% indicates a clear structural demand expansion
- Private hospital earnings expected to grow **5–7% annually**, driven by higher treatment intensity.
- Sector supported by aging population and Thailand's role as a **regional medical hub**
- Healthcare expenditure increasing from **~3.6% (historical) to ~6.0% of GDP by 2026** reflects a structural shift toward **healthcare-led economic contribution**

Costs & Access Pressures

- Medical inflation reached **10.8%** vs **0.7%** headline inflation, significantly increasing treatment costs
- Public hospital occupancy rose to **~77%** while private declined to **~58%** , amid weaker middle-income demand and reduced non-essential spending.
- Insurance structures are shifting toward **30–50%** co-payment models, as rising medical claims and cost pressures reshape reimbursement and patient cost-sharing.

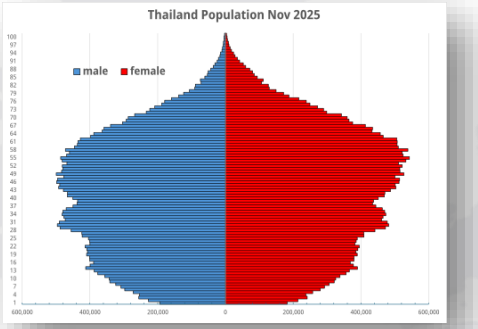
*Sources: Statista, Krungsri, KAsset, NHSO, **Statistical Yearbook Thailand 2025***

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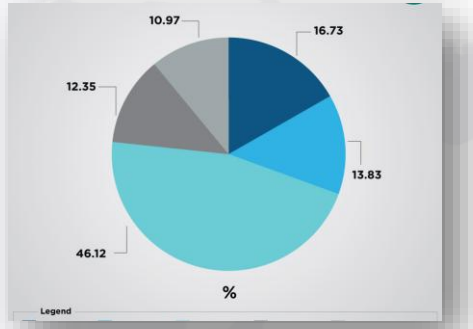
Social Outlook: Demographics, Disease Burden & Healthcare Access

Thailand is undergoing a rapid demographic transition into an ultra-aged society, significantly increasing long-term demand for healthcare services. Rising chronic disease burden and uneven healthcare access across regions are creating structural pressure on the system.









Aging Population

- Elderly population growing at **~4.4% CAGR (2020–2030)**, reaching **~21% by 2030 and ~32% by 2050**
- Elderly population expected to reach **~30% by 2040 (from 22.4% in 2026)**
- Aging base already exceeds youth population (**13.6m vs 9.5m**)

Demographic Shift

- Structural population shift, with **deaths (0.56m) exceeding births (0.54m)**
- Workforce contraction emerging as **retirees surpass new entrants (2023)**
- Urbanization at **~54.3%**, concentrating population in major cities

Disease Burden

- Non-communicable diseases (NCDs) dominate demand, driving **long-term treatment intensity**
- Mental health burden affecting **~13.4 million people**, especially among younger populations

Access & Inequality

- Healthcare workforce at **~1 doctor per 2,000 people vs WHO benchmark of 1:1,000**
- Strong regional disparity, with **~1:462 in Bangkok vs ~1:5,000 in underserved provinces**
- Access to advanced care remains concentrated in urban centers

Sources: GISTDA, Nation Thailand, Worldmeters

Fixed

Technological Outlook

Thailand's healthcare system is undergoing rapid digital and technological transformation, with HealthTech shifting from pandemic-driven adoption to a core structural growth driver.

Core Drivers

1 Digital Health Adoption

- **Telehealth app downloads** increased by approximately **20%**.
- **Around 2 million telehealth consultations** have been conducted.

2 Government & National Initiatives

- **10,628+ healthcare facilities** are connected to the **national Personal Health Record (PHR) system**.
- **THB 1.5 billion** has been invested to support **digital health infrastructure**.

3 Advanced Medical Technology & Diagnostics

- Hospitals investing in **AI diagnostics** and high-intensity equipment **(MRI, CT, robotics)** to support complex treatment demand

4 Infrastructure & Capacity

- System capacity at **~2.4 beds** per 1,000 population, with utilization gap of **~77% public** vs **~58% private**



Overview - How is technology reshaping healthcare industry?

- "Silver economy" expected to reach **trillions of baht**, with HealthTech as a key enabler of elderly care
- Telemedicine regulation and licensing frameworks are already in place, supporting **scalable digital healthcare delivery**
- Technology increasingly used for **financial auditing, automation, and system-wide efficiency improvements**

Sources: Ken Research, Finansia, Keungsri

Legal Outlook

Thailand's healthcare sector is governed by a structured and tightly regulated legal framework covering operations, licensing, and medical standards. Market entry is restricted under foreign ownership rules, with selective incentives supporting investment in high-value healthcare segments.

Healthcare Laws & Compliance Framework

1 Core Regulatory Structure

- Governed by key legislation including the **Medical Profession Act, Health Facility Act, and Medical Device Act**
- Defines standards for **professional conduct, facility operations, and medical equipment oversight**

2 Compliance & Updates

- Strict regulatory oversight across **clinical practices, infrastructure, and product safety**
- Ongoing updates (e.g., **2026 FDA changes**) strengthening reporting requirements and regulatory alignment

Foreign Investment & Ownership Restrictions

1 Ownership Framework

- Foreign ownership typically capped at **49% under the Foreign Business Act (FBA)**
- Healthcare classified under **FBA List 3**, requiring licensing for foreign participation

2 Investment Incentives

- BOI promotion enables **100% foreign ownership**, alongside **tax exemptions (up to 8 years)**
- Incentives focused on **high-tech healthcare, specialized hospitals, and medical manufacturing**

3 Market Approach

- **Mass-repurposing EV lines:** Retooling underutilized EV Gigafactories (e.g., LG, Samsung) to produce LFP cells for BESS to de-risk operations
- **Sourcing:** Leveraging friend-shoring partners* (e.g. Korea, Australia) to secure 10% Domestic Content Bonus

Licensing & Operational Requirements

1 Ownership Framework

- **Facility & Professional Licensing** Medical facilities require licensing from **Department of Health Service Support (DHSS)**
- Practitioners must hold valid licenses from **relevant Thai professional councils**

2 Investment Incentives

- Advanced equipment (e.g., **MRI, X-ray**) requires separate **FDA registration and approval**
- Recent regulations shifting toward **risk-based alignment between facility operations and product class**

Sources: thelegal.co.th, the thaiger, thaipbs

Environmental Outlook

Environmental factors are increasingly influencing healthcare demand in Thailand, with pollution and climate patterns driving seasonal disease spikes. Tightening sustainability regulations are also raising compliance requirements and cost pressures for healthcare providers.





Environmental Health Drivers

- PM2.5 pollution contributing to **rising respiratory and cardiovascular cases**, with seasonal surges observed in Bangkok and Northern Thailand
- Climate-related diseases such as dengue linked to **~5% growth in outpatient (OPD) volume**, driven by changing rainfall patterns
- Environmental conditions creating **recurring and predictable demand cycles**, particularly for respiratory and infectious diseases

Sustainability & Regulatory Pressures

- "**GREEN & CLEAN Hospital"** initiative enforcing stricter **medical waste disposal and wastewater treatment standards** to maintain operating licenses
- Transition toward **2050 carbon neutrality** requiring investments in facility upgrades and emissions reduction
- **ESG focus** driving adoption of **solar energy and energy-efficient systems**, as hospitals align with investor expectations and manage long-term costs

Sources: pic crd International Finance, Bangkok Post, NESDC, Finansia

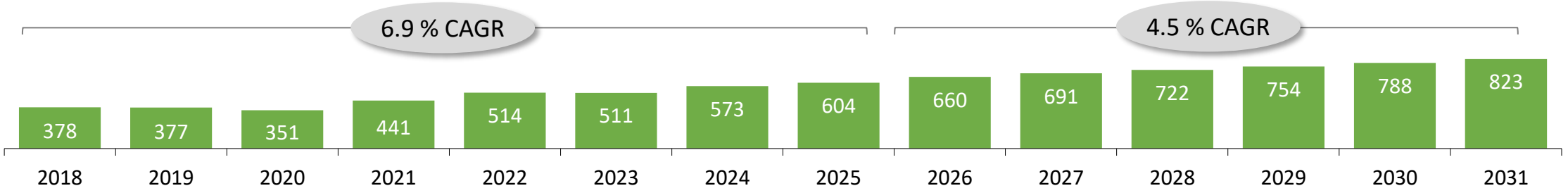
Macroeconomics

Singapore's GDP is projected to expand at a CAGR of 4.5% over 2026–2031 and to maintain stable medium-term growth, supported by continued investment in technology, advanced manufacturing, and financial services.



Socio-economic Statistics	
GDP (2025)	USD 577 Bn
GDP / Capita (2025)	USD 99,365
GDP Growth (2026-2032)	4.5 % CAGR
Population (2025)	6.11 Mn
Urbanisation (2025)	100%
Inflation rate (2025)	0.9%

Singapore GDP – Historical and Forecasts (USD Bn)



- **Singapore's GDP** is expanded **6.0% YoY in 1Q26**, extending the **5.7% growth recorded in 4Q25**, driven primarily by AI-related demand across electronics, precision engineering, and wholesale trade..
- Continued capital inflows, strong financial sector performance, and Singapore's position **as a regional safe-haven hub** support a favorable business environment despite external geopolitical uncertainties.

Sources: International Monetary Fund (IMF), Statista, iFastglobal markets

Demographic & Culture

Singapore's 2026 market is a super-aging, hyper-connected society defined by a deeply ingrained lifestyle of personal accountability and rapid digital health transformation.

1 Preventative Culture

Singapore's efficiency-driven culture supports rapid adoption of **national wellness initiatives like Healthier SG**.

2 Super-Aged Society

With over 21% aged 65+, demand is shifting toward **automated senior care** and **longevity wellness**.

3 Wellness Trends

Tech-dependent urban consumers favor **personalized wellness apps**, **health monitoring**, and **preventive lifestyles**.



4 Healthcare Spending

Public healthcare spending has **surpassed $20 billion amid infrastructure expansion** and **rising medical costs**.

5 Economic Landscape

Demographic shifts are **driving functional nutrition** and **senior wellness for healthier, independent aging**.

6 Digital Health Hub

Universal 5G connects Singapore's cloud-based healthcare ecosystem, reinforcing its regional medtech leadership.

Source: MOH, Straits Times, Maybank, BioSpectrum Asia, Global Wellness Summit, Trade.gov

Total Household Spending

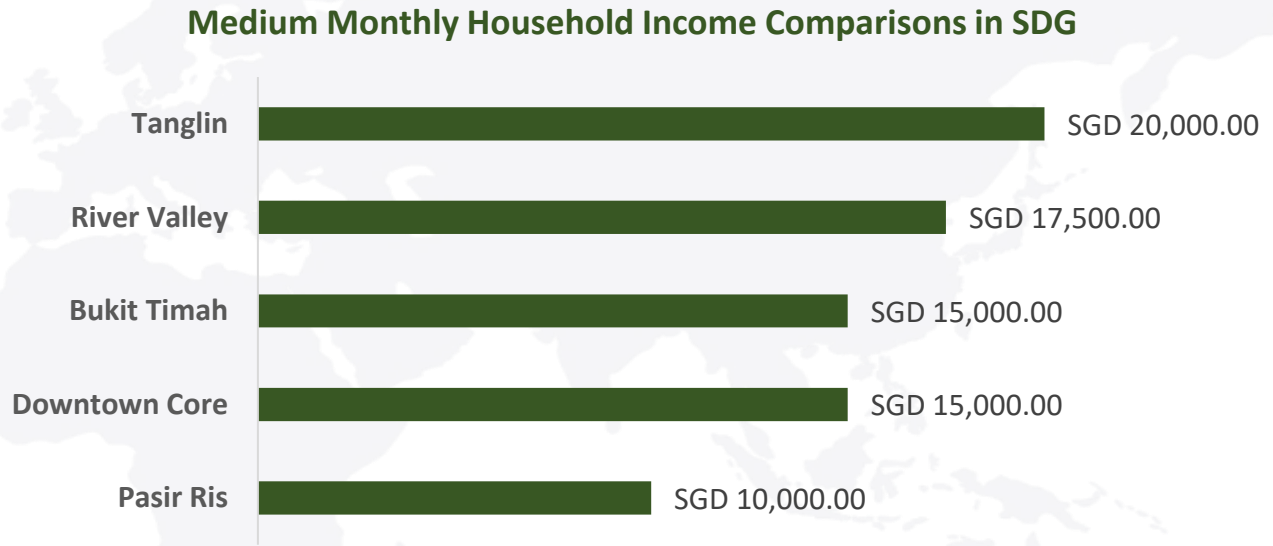
Singapore's affluent districts exhibit substantial income concentration, creating distinct premium and mass-market consumer segments that support differentiated market positioning and pricing strategies.

National Monthly Household Expenditure

SDG 5,931

CAGR Growth of Household Expenditure

2.8 %

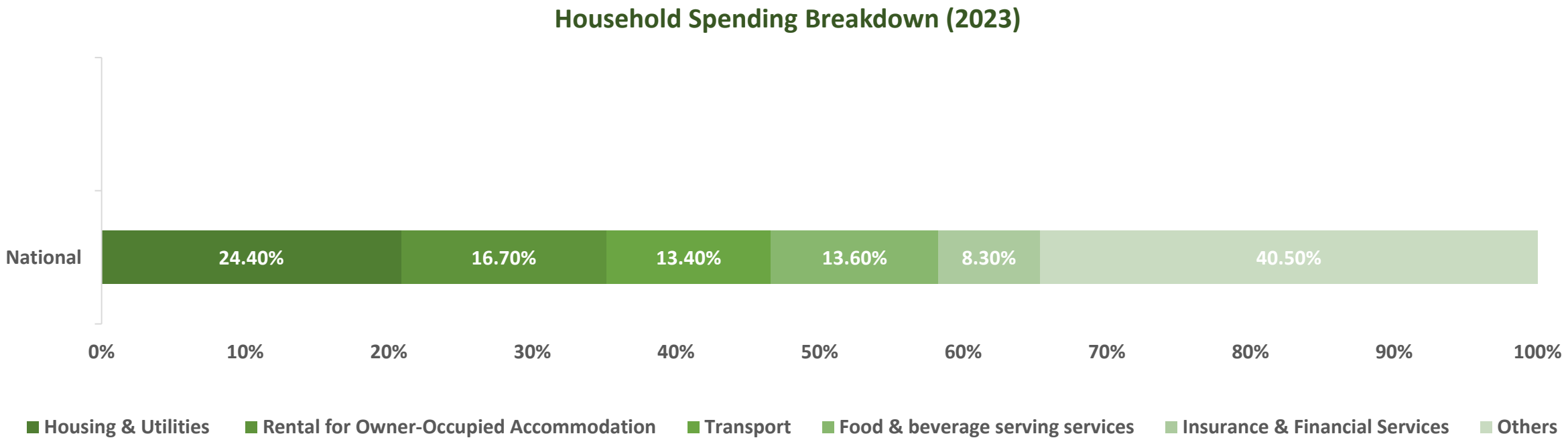


- **Tanglin's median household income (SGD 20,000**) is approximately 100% higher than **Pasir Ris (SGD 10,000)**, highlighting a significant premium consumer segment within Singapore.
- Four of the districts exceed **SGD 15,000** in monthly household income, indicating a broad affluent consumer base beyond Singapore's traditional luxury enclaves.

Sources: Singstat.gov.sg, seedly.sg

Total Household Spending Breakdown

Singapore's spending profile reflects a mature, high-income economy, with household budgets heavily allocated toward housing, mobility, and financial security rather than basic necessities.



- **Housing-related expenditure** accounts for approximately **35% of total household spending**, reinforcing housing as the largest component of household budgets.
- **Housing, transport**, and **dining-related expenditure collectively represent roughly 55–60% of spending**, highlighting the importance of mobility and lifestyle services in daily consumption.
- Insurance and financial services account for **approximately 7% of expenditure**, reflecting Singapore's strong savings culture and demand for wealth protection products.

Sources: Singapore Dept of Statistics

Healthcare Value Chain

Singapore captures value across public care, premium services, and medtech, despite capacity and affordability pressures.

1 Policy & regulation

- MINISTRY OF HEALTH SINGAPORE
- HSA Health Sciences Authority
- EDB: SINGAPORE
- aic agency for integrated care
- Health Promotion Board

2 Payers

- Medisave
- Medifund
- MediShieldLife

- Subsidies
- Integrated Shield Plans
- Employer benefits

3 Providers

- SingHealth
- THOMSON MEDICAL
- NUHS National University Health System
- NHG Health
- RafflesMedical Your Trusted Partner for Health
- IHH Healthcare

- Polyclinics

4 Inputs

- Agency for Science, Technology and Research SINGAPORE
- NUS National University of Singapore
- DukeNUS Medical School
- synapxe

- Global pharma/medtech
- AI/health-tech

5 Access / customers

- Residents
- IP holders
- Elderly/chronic patients
- Expats
- Regional premium patients

Where commercial value is captured

Public clusters	Premium private care	Biomedical / medtech	Aging care / digital productivity
Capture national care delivery, specialist centers, and population health management	IHH/Parkway, Raffles, and Thomson capture insured, expat, corporate, and premium patient demand	Capture high-value HQ, R&D, manufacturing, and clinical innovation activity	Aging care and digital productivity are becoming major value pools

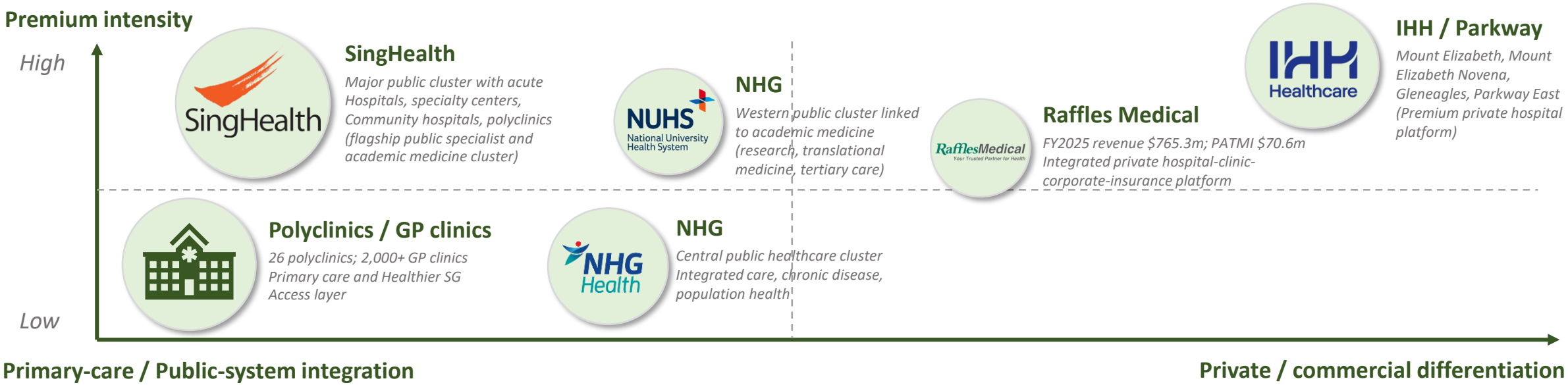
Main value-chain bottlenecks

Manpower scarcity	Aging burden	Land constraints	Insurance inflation
Drives need for AI, automation, and workflow redesign	Raises long-term care, rehab, and chronic disease burden	Limit physical capacity expansion	Can pressure affordability and private care demand

Sources: Sagasia internal research based on Thailand BOI Healthcare and Wellness Industry, MOPH FY2025 Action Plan, TDRI Universal Healthcare analysis, and Krungsri Research healthcare industry outlooks.

Providers Landscape

Singapore's provider market is split between integrated public care and premium private differentiation.



Market structure

Public clusters

- National / regional care
- Specialist centers
- Population health

Polyclinics / GP clinics

- 26 polyclinics; 2,000+ GP clinics
- Primary care and Healthier SG access layer

IHH / Parkway

- Premium private hospital platform

Raffles Medical

- Integrated private care
- Corporate and insurance platform

Strategic reading

- **Market is organized**, not fragmented
- **Public integration** vs **private premium differentiation** is the key split
- **Corporate healthcare** and **regional patient trust** drive private competition
- **Digital / AI** and **aging care** are increasingly central to provider strategy

Sources: Sagasia internal research based on Singapore MOH public healthcare cluster materials, IHH Healthcare reports, Raffles Medical Group FY2025 announcement, and Singapore MOH healthcare capacity expansion materials.

Customer Segments & Demand-Side View

Singapore's healthcare demand is driven less by population size and more by high-value needs, supported by aging, affluent consumers, insurance coverage, medical trust, and preventive-care reform.

Key Customer Segments & Demand

Subsidized Public Patients
Affordable public care, chronic disease management, specialist referrals

Integrated Shield Plan Holders
Higher ward class, private hospitals access, doctor choice

Elderly Residents
Chronic care, rehab, nursing homes, community/ home care

Healthier SG enrollees
Family doctor, prevention, screening, vaccines, lifestyle management

Corporate/ Expat Patients
Convenient private GP/specialists care, insurance acceptance

Regional Premium Patients
Complex care, safety, trust, advanced medicine

Likely Provider Choice

Subsidized Public Patients	Public Clusters, Poly Clinics, Public Hospitals
Integrated Shield Plan Holders	IHH/Parkway, Raffles, Private Specialists, Public A/B wards
Elderly Residents	Public Clusters, AIC-linked providers, community hospitals
Healthier SG enrollees	Healthier SG/ GP Clinics, Polyclinics
Corporate/ Expat Patients	Raffles,IHH/ Parkway, Corporate Clinics
Regional Premium Patients	IHH Parkway, Raffles, Public/ Private Specialists

Quantitative demand signals

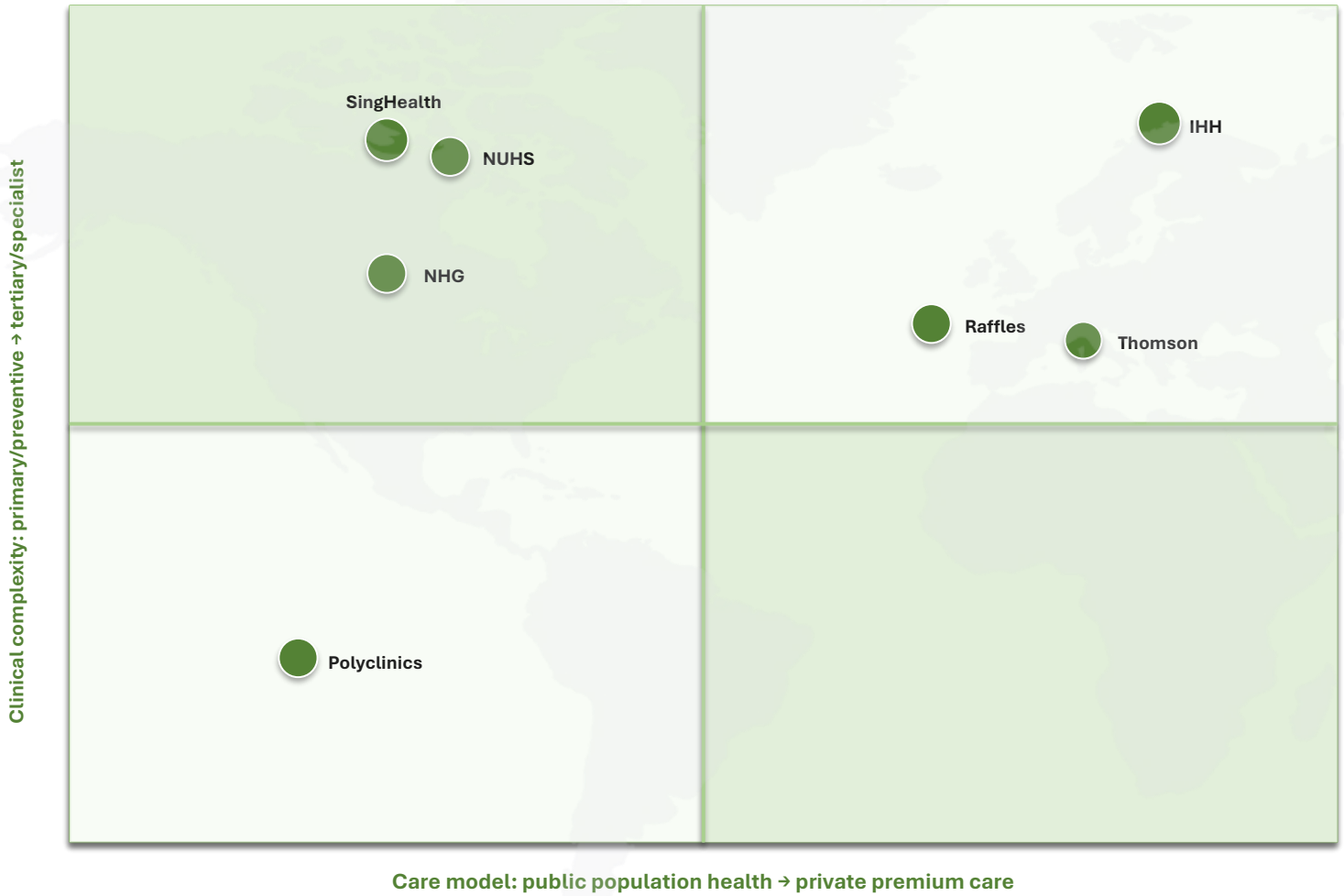
- **Universal coverage:** 100% public insurance coverage, with 69% Integrated Shield Plan penetration.
- **Preventive-care adoption:** Healthier SG enrolment exceeded 1.3 million residents by 2025.
- **Capacity expansion:** MOH plans to add 13,600 beds between 2025 and 2030.

Provider implications

- **Insurance-driven demand:** Private care relies on insurance and employer benefits.
- **Preventive-care shift:** Healthier SG favors primary care, prevention, and digital records.
- **Productivity pressure:** Manpower shortages accelerate AI and care redesign.

Sources: Sagasia internal research based on Commonwealth Fund Singapore Health System Profile 2026, MOH Healthier SG White Paper, MOH Committee of Supply 2025, and MOH healthcare capacity expansion materials.

Brand Positioning Axis



Entry barriers

- Very high cost of land, labor, specialists, technology, and compliance.
- Strict regulatory and quality expectations create high operating standards.
- Public clusters already dominate much of national care delivery.
- Manpower scarcity makes new capacity difficult to staff.

Provider moats

- SingHealth/NHG/NUHS: national mandate, integration, specialty centers, academic depth.
- IHH/Parkway: premium brands, specialist access, expat and regional trust.
- Raffles: integrated clinic-hospital-insurance-corporate platform.
- Thomson: maternity, fertility, women's and family-health brand.

Success factors

- Deliver clinical excellence, safety, and trust at very high standards.
- Integrate with Healthier SG, digital records, and population-health pathways.
- Use AI/productivity tools to offset labor constraints.
- Build aging-care capacity: rehab, dementia, home care, step-down care.

Strategic reading

Singapore is structurally different: public clusters manage population health while private groups differentiate through premium, insured and regional specialist care.

Sources: Sagasia internal research based on Singapore MOH public healthcare cluster materials, IHH reports, Raffles FY2025 announcement, Singapore EDB healthcare materials, and U.S. ITA Singapore Healthcare Guide.

Political Outlook

Singapore's stable governance and strong fiscal position continue to support long-term policy execution and investment confidence. Healthcare remains a strategic priority as the country enters a super-aged society in 2026 and accelerates preventive care reforms.

Healthier SG
1.3 Mn Residents Enrolled

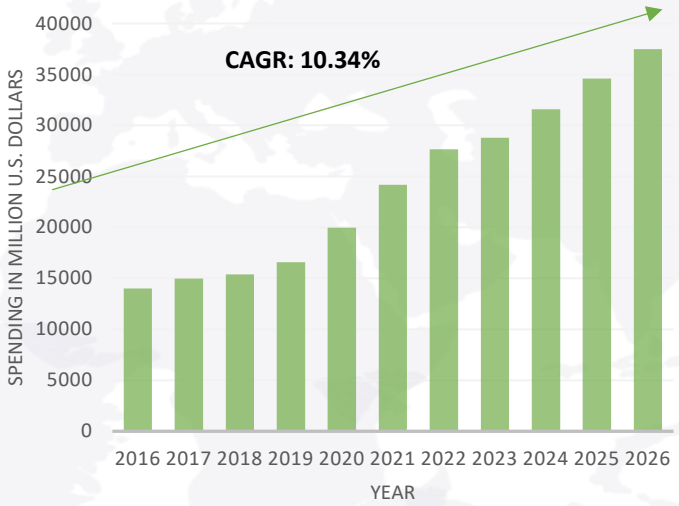
Strategic Priorities

- Singapore enters super-aged status in 2026, with over 21% of residents aged 65+, increasing healthcare and social support needs
- Government priorities remain focused on economic competitiveness, innovation, and long-term fiscal sustainability

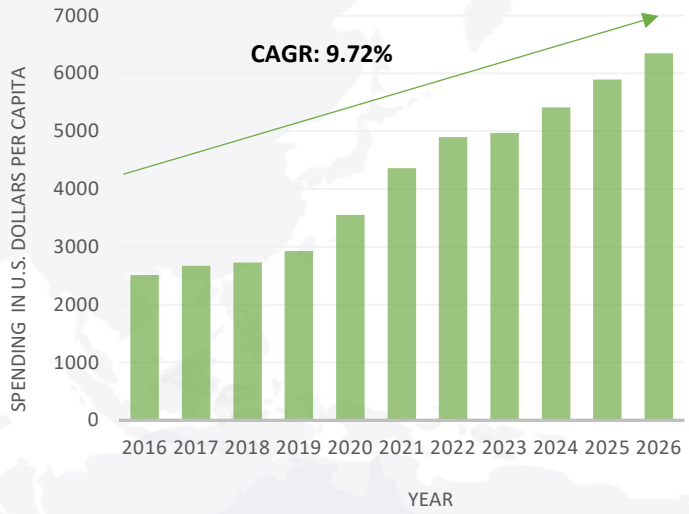
Healthcare System Reforms

- Healthier SG has enrolled over 1.3 million residents, accelerating the shift toward preventive and community-based care
- Plans for 32 polyclinics by 2030 and 31,000+ beds over the next decade to support rising healthcare demand

Expenditure on Healthcare in Singapore from 2016-2026



Spending per Capita on Healthcare Expenditure in Singapore from 2016-2026



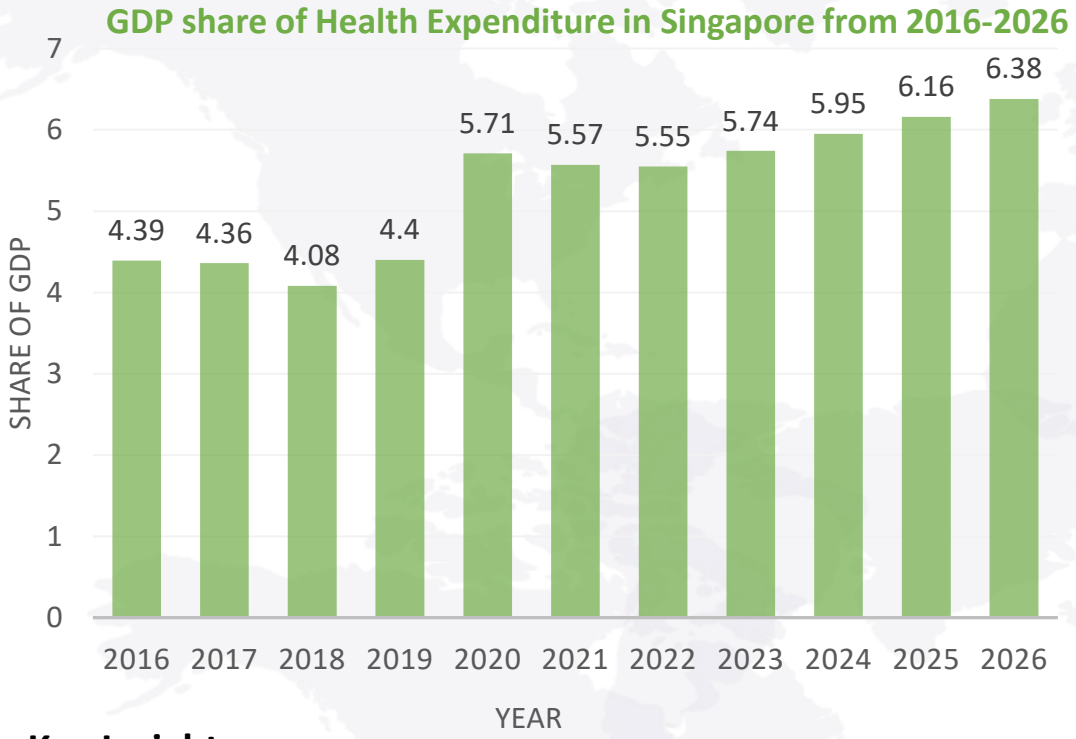
Policy Environment

- Healthcare received **S$22.5bn in FY2026 (+10.4% YoY)**, while development expenditure increased **31% to S$2.47bn** for hospitals, digital health, and research.
- Healthier SG and CareShield Life reforms continue to strengthen preventive care, long-term care, and healthcare affordability

Sources: Statista, MOH, DBS, Strait Times

Economic Outlook

Singapore's healthcare sector continues to outpace overall economic growth, driven by ageing demographics and sustained government investment. However, rising medical inflation and insurance cost pressures are reshaping affordability and patient cost-sharing dynamics.



- Healthcare spending is projected to increase **10.4%** in **FY2026,** significantly outpacing **GDP growth of 1.8–3.0%**
- National healthcare expenditure is expected to rise from **5.9% of GDP** today **to 9.0% by 2030**
- Singapore's **super-aged population** and **growing chronic disease burden** continue to drive long-term healthcare demand
- As a regional medical hub, Singapore attracts nearly **500,000 international** patients annually and hosts **13 JCI-accredited facilities**

Key Insights

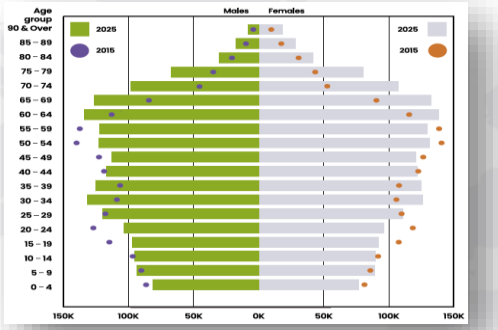
- Private medical inflation is projected to reach a record 16.9% in 2026, significantly increasing treatment costs and insurance claims across the healthcare system
- New Integrated Shield Plan reforms will increase patient cost-sharing, with annual co-payment caps doubling from S$3,000 to S$6,000 to manage rising medical inflation
- Healthcare delivery is gradually shifting toward outpatient, home-based, and telecare models as ageing demographics and capacity constraints reshape care provision

Sources: Statista, DBS group research, OCBC, UOB Kay Hian

Social Outlook: Demographics, Disease Burden & Healthcare Access

Singapore officially became a super-aged society in 2026, with seniors accounting for more than one-fifth of the population. Rising chronic disease prevalence and growing elderly care needs are increasing long-term demand for healthcare and community-based services.









Aging Population

- Singapore attained **super-aged status in 2026**, with 21% aged 65+ and 25% expected by 2030
- The old-age **dependency ratio** has **fallen to 2.7 working-age adults per senior**
- Longer life expectancy and lower fertility rates continue to accelerate population ageing

Demographic Shift

- Singapore is shifting focus to **age-friendly communities** and **ageing-in-place**
- Healthcare planning increasingly focuses on transforming **HDB estates** into integrated **"Health Districts“**
- A shrinking workforce may increase pressure on healthcare staffing and productivity

Disease Burden

- Cardiovascular diseases and cancers account for **over 43% of the increase in DALYs**
- **Type II diabetes, hypertension, and hyperlipidaemia** continue to drive healthcare demand
- **Mental health** concerns are rising, with poor mental health prevalence reached 17%

Access & Inclusivity

- **Over 90% of residents** live within 4km of a hospital, supporting strong healthcare access
- Healthcare disparities are increasingly **socioeconomic rather than geographic** despite broad coverage
- **CHAS expansion and community-based care** support healthcare access for seniors in heartland areas

Sources: Department of statistics, MOH, polyclinic

Technological Outlook

Singapore is integrating AI and smart healthcare infrastructure into routine care delivery, positioning technology as a key enabler of healthcare sustainability and productivity.

Core Drivers

1

Digital Health Adoption

Over 60% of Singaporeans use digital health tools, with **HealthHub** serving as the national healthcare platform.

2

National Digital Health Infrastructure

NGEMR and **NEHR** now support seamless data sharing across **Singapore's public healthcare system**.

3

AI & Precision Medicine

National AI initiatives and **precision medicine programs** are expanding the use of predictive analytics and data-driven healthcare.

4

Smart Capacity Expansion

Woodlands Health Campus added **1,400 beds**, while **IoT-enabled Hospital-to-Home programs** support remote patient monitoring.



Overview - How is technology reshaping healthcare industry?

- **Healthcare is one of four National AI Missions,** supporting diagnostics, predictive analytics, and chronic disease management
- **Telemedicine adoption** continues to expand, with major platforms reporting **approximately 20% annual growth in consultations**
- Technology is supporting the shift **from hospital-centric care toward preventive and community-based healthcare**

Sources: ITU.INT pic crd, UOB Kay Hian, Morgan Stanley

Legal Outlook

Recent regulatory reforms have reshaped Singapore's healthcare landscape, with the Healthcare Services Act, Health Information Act, and AI governance frameworks establishing new standards for healthcare delivery and digital health innovation.

Healthcare Laws & Compliance Framework

1 Healthcare Services Act (HCSA)

- Fully replaced PHMCA in 2026 with a service-based licensing framework
- Covers hospitals, telemedicine, nursing homes, and community care services

2 Health Information Act (HIA)

- Mandates provider participation in the National Electronic Health Record (NEHR)
- Introduces stricter health data governance and cybersecurity requirements

Licensing & Operational Requirements

1 Healthcare Licensing

- 16 licensable healthcare services regulated under the HCSA framework
- Applications managed through the Healthcare Application & Licensing Portal (HALP)

2 Professional Standards

- Licensed providers must appoint designated governance personnel under HCSA
- Practitioners must maintain registration with relevant professional boards

3 Health Product Regulation

- Medical devices and pharmaceuticals are regulated by the Health Sciences Authority (HSA)
- Importers and manufacturers require dealer licences and GDP compliance

Digital Health & Market Access

1 AI & Digital Health Regulation

- AIHGle 2.0 establishes accountability standards for clinical AI deployment
- New guidance clarifies governance requirements for Generative AI in healthcare

2 Foreign Investment Framework

- Regulatory sandboxes support testing of emerging healthcare technologies
- Singapore positions itself as a regional hub for digital health and medtech innovation

Sources: moh.gov.sg, hcsa.gov.sg, medinex

Environmental Outlook

Environmental pressures are increasingly shaping healthcare demand in Singapore, while stricter sustainability requirements are driving investment in greener and more energy-efficient healthcare infrastructure..



Climate & Public Health Risks

- **Rising temperatures** are increasing demand for **heat-related illness** treatment and respiratory care, with temperatures projected to rise by **2.5°C–5.8°C** by end-century
- **Changing rainfall patterns** continue to contribute to dengue outbreaks, requiring hospitals to maintain surge capacity and diagnostic preparedness
- Recurring transboundary haze episodes periodically increase outpatient visits **for asthma, COPD, and other respiratory conditions**



Sustainability & Regulatory Pressures

- **Mandatory climate-related disclosures** aligned with **ISSB standards** require listed healthcare providers to report **Scope 1, 2, and 3 emissions**
- All new public hospitals are required to achieve **BCA Green Mark Platinum standards**, driving investments in energy-efficient systems and solar adoption
- **Singapore's carbon tax** will rise from **S$25/tCO₂e to S$45/tCO₂e by 2027**, increasing cost pressures for energy-intensive healthcare facilities

Sources: pic crd archdaily, The Straits Times, greeenplan.gov.sg

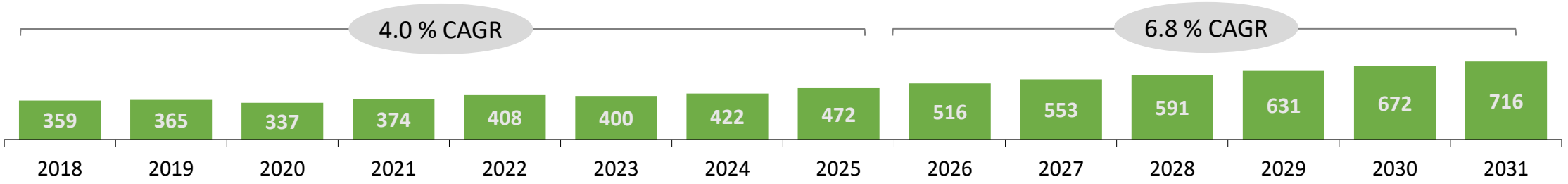
Macroeconomics

Malaysia's economy is projected to grow 4.6%–4.9% annually over the next three years, supported by household consumption, AI-related investments, and technology exports.



Socio-economic Statistics	
GDP (2025)	USD 472 Bn
GDP / Capita (2025)	USD 13,950
GDP Growth (2026-2032)	6.8 % CAGR
Population (2025)	34.3 Mn
Urbanisation (2025)	79.8 %
Inflation rate (2025)	1.4 %

Malaysia GDP – Historical and Forecasts (USD Bn)



- Malaysia's status as a **net energy exporter** and the presence of broad government subsidies help cushion the economy from global energy price volatility..
- Strong banking fundamentals, supported by **10.9% SME financing growth and projected NPL ratios** of only **~1.6%** by end-2027, contribute to a favorable business environment despite elevated geopolitical uncertainties..

Sources: International Monetary Fund (IMF), Statista, Department of Statistics Malaysia

Demographic & Culture

Malaysia's 2026 market is a modernizing, value-driven economy powered by resilient domestic consumption and a major state-backed push into global medical tourism.



1 MADANI Culture

Local consumer culture focuses heavily on community equity and digital-first habits, driven by the state's national *Ekonomi MADANI* **("Civil Economy")** framework



2 Resilient Economy

The economy maintains solid momentum with a **stable 4%–5% GDP** growth forecast, strongly anchored by robust domestic consumer demand.



3 Cost-Conscious Trends

Faced with **cost-of-living anxieties**, local urban consumers actively adjust their habits by seeking practical, value-driven lifestyle options.



4 Medical Tourism

Driven by ***Visit Malaysia Year 2026***, high-value healthcare travel spending is surging as international patients flock to private facilities.



5 Healthcare Inflation

Domestic medical inflation **is rising toward 16%,** forcing private insurers and employers to completely restructure co-payment policies.



6 Social Commerce Hub

The digital marketplace is expanding rapidly, with social platforms serving as primary storefronts for lifestyle and wellness retail.

Sources: MOF Malaysia, Bank Negara, Vodus, MHTC, The Star

Total Household Spending

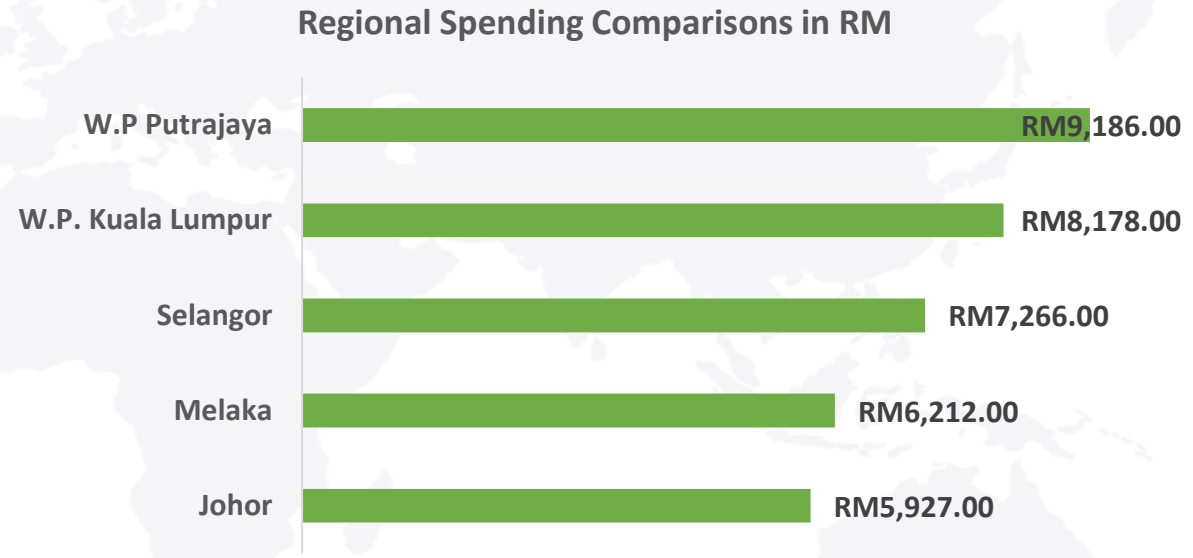
Malaysia's household spending is concentrated in the Greater Klang Valley and administrative capital regions, highlighting strong purchasing power in urban centers while maintaining sizeable consumer markets across secondary states.

National Monthly Household Expenditure

RM 5,566

CAGR Growth of Household Expenditure

3.9 %

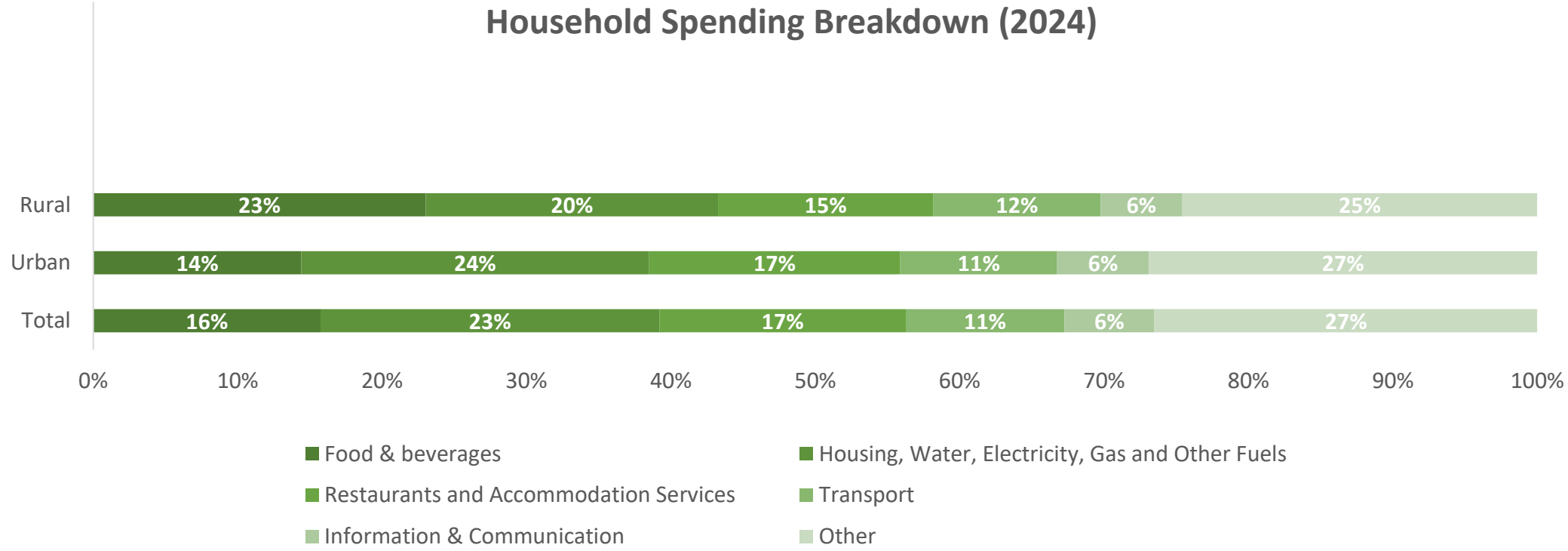


- **W.P. Putrajaya, Kuala Lumpur, and Selango**r demonstrate strong purchasing power, reinforcing their position as Malaysia's key consumer and economic hubs.
- The presence of multiple regions with household expenditure above RM7,000 suggests that affluent demand extends beyond a single city and is supported by a broader urban ecosystem.

Sources: : Department of statistics Malaysia, Worldbank

Total Household Spending Breakdown

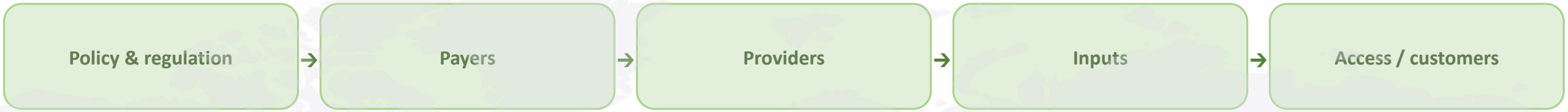
Malaysia's household spending remains concentrated in essential categories, with housing, food, and dining accounting for the majority of expenditure, reflecting a consumption pattern driven by daily necessities and urban living costs.



- **Housing and utilities** account for approximately **24–27% of household spending** across all segments, making them the single largest expenditure category.
- **Urban households** allocate a larger share of spending **to transport (~11%) and information & communication (~6%)** than rural households, reflecting more connected and service-oriented lifestyles.
- Food, housing, and restaurant-related expenditure collectively represent **approximately 56–58% of total household budgets**, highlighting resilient demand for essential consumer sectors.

Sources: Department of statistics Malaysia

Healthcare Value Chain



Value chain layer	Key players / components
Policy & regulation	MOH Malaysia, MDA, MIDA, MHTC
Payers	MOH budget/subsidies, private insurance, employer benefits, OOP, foreign medical tourists
Providers	MOH hospitals/Klinik Kesihatan; IHH, KPJ, Sunway, Columbia Asia, Island/Gleneagles/Pantai
Inputs	Pharmaniaga, Duopharma, local/MNC pharma, Top Glove, Hartalega, global device firms
Access / customers	Public patients, domestic private, corporate, Indonesian/regional medical tourists, elderly/NCD patients

Where commercial value is captured

- IHH, Gleneagles, Pantai, Prince Court, and Island capture premium/private and medical tourism demand.
- KPJ and Columbia Asia capture domestic private and middle-class demand.
- Sunway captures high-growth integrated private care tied to its broader ecosystem.
- Medtech manufacturers capture export/manufacturing value beyond healthcare services.

Main value-chain bottlenecks

- Public hospital crowding and financing pressure.
- Public-private workforce competition for doctors, nurses, and specialists.
- Insurance affordability can limit private hospital access.
- Medical tourism exposed to regional competition and source-market shocks.

Sources: Sagasia internal research based on Malaysia MOH Health Facts 2025, Malaysia Health White Paper, Malaysia Healthcare Travel Council materials, and MIDA medical technology / medical devices materials.

Provider & Competitive Landscape

Market structure

Malaysia's provider market is a dual-track system. Public hospitals and clinics dominate national access, while private providers compete on speed, specialist care, medical tourism, and middle-class/insured patient demand.

Provider / group	Quantitative scale / hard numbers	Competitive positioning
MOH public system	150 hospitals/special institutions; 46,855 beds; 1,131 health clinics	Subsidized national access and primary care backbone
IHH Malaysia	Island deal made Island IHH's 18th Malaysia hospital	Premium/private platform and medical tourism champion
Island Hospital	600 beds; 80 specialists	Penang medical tourism anchor
KPJ Healthcare	Targeting 6,000 beds by 2029 in sector reporting	Domestic private hospital network
Sunway Healthcare	1,805 licensed beds; FY2024 revenue RM1.85bn; net profit RM257.5m	Fast-growing integrated challenger
Columbia Asia	Community/suburban private hospital chain	Accessible middle-market private care

Strategic reading

Malaysia's competitive logic is cost-quality balance. Premium medical tourism players need trust and specialist depth, while domestic private players need accessibility, payer relationships, and operational efficiency.

Sources: Sagasia internal research based on Malaysia MOH Health Facts 2025, Reuters report on IHH-Island Hospital and Sunway Healthcare IPO, MIDF Malaysia Healthcare thematic report, and IHH Healthcare investor materials.

Customer Segments & Demand-Side View

Demand-side context

Malaysia's demand base combines high-volume public care with commercially meaningful private and foreign-patient demand. The public system handles mass affordability, while private hospitals serve domestic middle-class, insured, corporate, and medical tourism patients seeking affordable quality care.

Customer segment	Core demand / need	Likely provider choice
Public subsidized patients	Low-cost public treatment, primary care, specialist referrals	MOH hospitals, Klinik Kesihatan
Domestic private patients	Shorter waits, private rooms, diagnostics, specialists	KPJ, Sunway, Columbia Asia, Pantai, Gleneagles
Private insured patients	Covered private hospital and specialist treatment	IHH, KPJ, Sunway, private specialists
Corporate patients	Screening, outpatient panels, employee healthcare	Private clinics, corporate panels, private hospitals
Foreign medical tourists	Affordable quality care, English service, regional access	Island, Gleneagles, Pantai, Mahkota, Penang/KL/Melaka hospitals
Elderly / NCD patients	Chronic care, medicines, diagnostics, rehab	Public/private hospitals, pharmacies

Quantitative demand signals

- Government hospital admissions: 2.58m in 2024.
- MOH outpatient attendances: 84.88m in 2024.
- Healthcare travellers: 1.6m in 2024; healthcare travel revenue RM2.72bn in 2024; around RM3.3bn in 2025.

Provider implications

- Public demand remains huge, but private hospitals benefit from waits and service expectations.
- Medical tourism providers need specialist packages and international-patient operations.
- Domestic private providers need payer partnerships, accessible pricing, and credible specialists.

Sources: Sagasia internal research based on Malaysia MOH Health Facts 2025, Malaysia Healthcare Travel Council materials, TTG Asia medical tourism reporting, and Malaysian business media coverage of healthcare travel revenue.

Brand Positioning Axis



Strategic reading

Malaysia's provider market is a dual public-private system: IHH leads premium/medical tourism, KPJ leads domestic network scale, and Sunway is the high-growth challenger.

Entry barriers

- Established incumbents already occupy premium, domestic, and medical-tourism niches.
- Medical tourism reputation requires accreditation, outcomes, service channels, and foreign-patient trust.
- Specialist access is difficult because public/private systems compete for talent.
- Insurance affordability affects private demand depth.

Provider moats

- IHH: premium brands, regional platform, specialist depth, medical tourism channels.
- Island: Penang reputation, Indonesian/regional patient flows, specialist base.
- KPJ: domestic network scale and Malaysian patient familiarity.
- Sunway: integrated ecosystem, premium positioning, expansion capital.

Success factors

- Balance cost and quality: Malaysia must remain more affordable than Singapore while clinically credible.
- Build specialist packages for medical tourists.
- Develop payer/corporate relationships for domestic private demand.
- Use location strategy: Penang, KL/Klang Valley, Melaka, Johor each serve different flows.

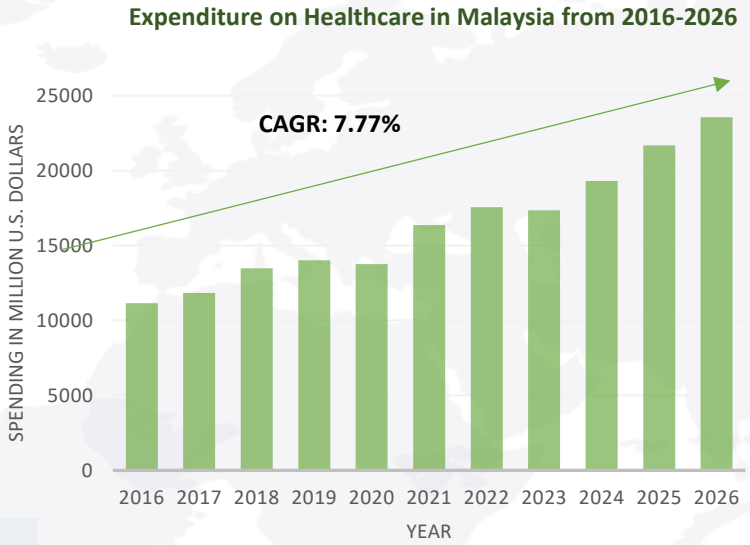
Sources: Sagasia internal research based on Reuters healthcare transaction reporting, MIDF Malaysia Healthcare thematic report, Malaysia Healthcare Travel Council materials, and MIDA medical-device industry materials.

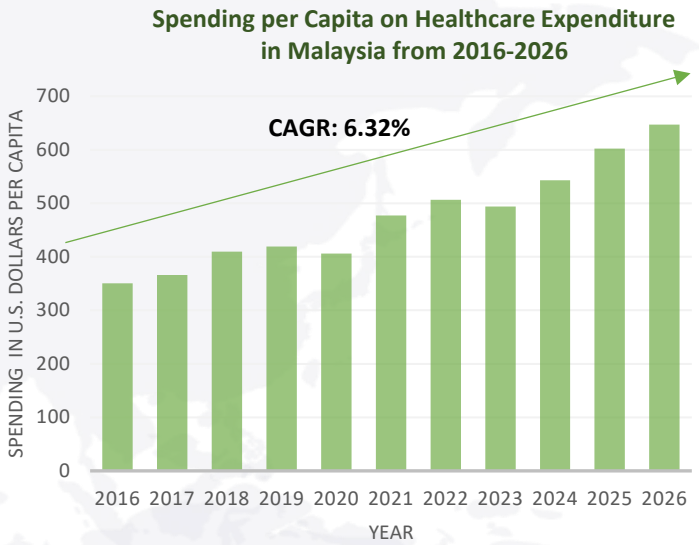
Political Outlook

Malaysia's relatively stable unity government continues to support policy continuity and healthcare sector development. Ongoing reforms are focused on healthcare financing, digitalization, and improving system efficiency under the Health White Paper.

Medical Tourism Year 2026

RM 12 Bn Revenue Target by 2030





National Priorities	• **The 13th Malaysia Plan (2026–2030**) prioritizes economic development under MADANI framework • **Visit Malaysia Year 2026 and Medical Tourism Year 2026** are expected to support investment, tourism, and healthcare activity
Macro Spillover into Healthcare	• The Health White Paper outlines **a 15-year healthcare transformation roadmap through 2038** • **DRG payments, EMR rollout**, and digital integration aim to improve efficiency and cost transparency

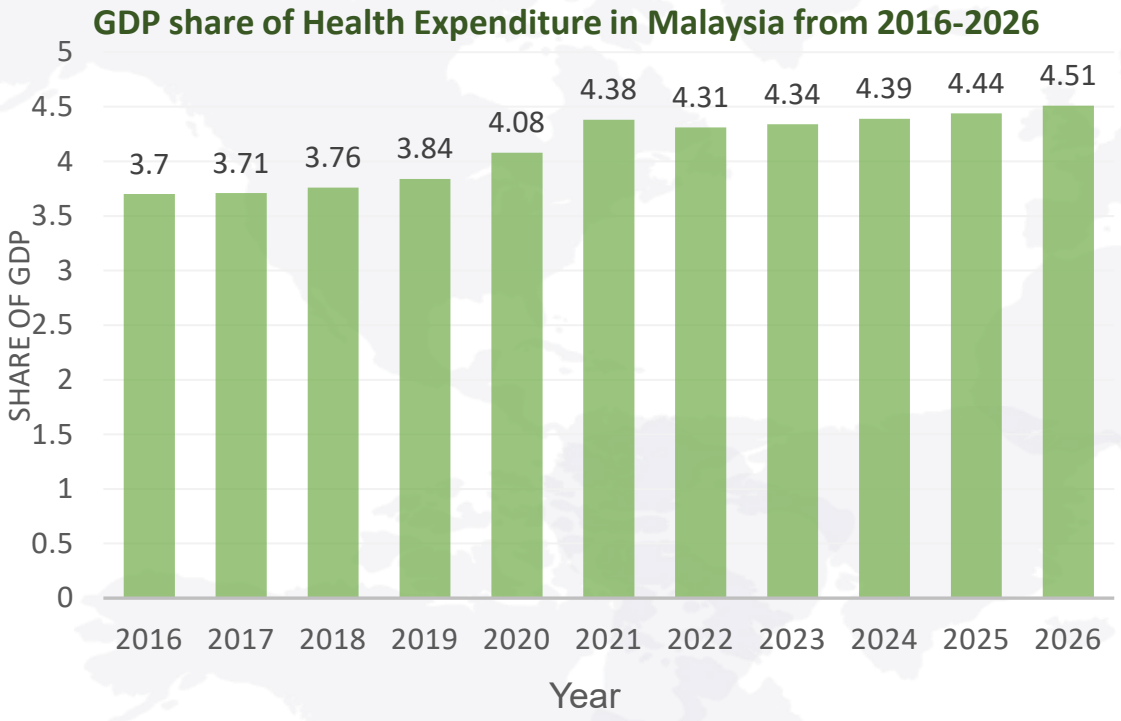
Policy Environment

- **Budget 2026** allocated **RM46.5bn** to the Ministry of Health**, up 2.8% from 2025**
- The New Investment Incentive Framework supports **FDI into health-tech, pharmaceuticals, and high-value industries**

Sources: MOH health white paper, Statista, MOF

Economic Outlook

Steady economic growth and extensive public healthcare subsidies continue to support healthcare access across Malaysia. Rising treatment costs and insurance claims, however, are placing increasing pressure on both public funding and household healthcare spending.



- Healthcare expenditure remains stable at **4.5–4.6% of GDP**, supported by **strong public subsidies and rising healthcare demand**
- Public healthcare remains highly accessible through **extensive government subsidies**, supporting sustained healthcare demand across income groups
- **An ageing population** and rising prevalence of non-communicable diseases **(NCDs)** continue to drive long-term healthcare demand
- **Budget 2026** allocated **RM46.5bn** to healthcare, supporting service expansion, workforce development, and healthcare accessibility

Key Insights

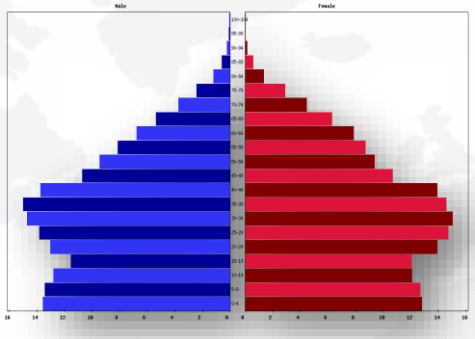
- Medical inflation is projected to reach 10–15% in 2026, significantly exceeding the Ministry of Health's 2.8% budget increase and creating a widening healthcare funding gap
- Out-of-pocket spending remains high at 35–37% of total healthcare financing, while rising treatment complexity and 56% medical claims inflation (2021–2023) continue to pressure affordability
- Reforms such as MyPriMe, DRG-based payments, and RM60mn support for affordable insurance products aim to improve transparency and accessibility

Sources: Bank Negara Malaysia, Statistsa,The Star, worldbank

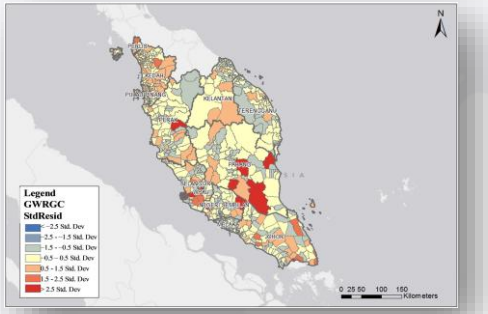
Social Outlook: Demographics, Disease Burden & Healthcare Access

Malaysia is transitioning toward an aged society, with a growing elderly population and slowing population growth. Rising NCD prevalence and persistent urban-rural healthcare disparities are increasing pressure on the healthcare system.









Aging Population

- Older adults aged **65+ account for over 8%** of Malaysia's population, projecting to be an **aged society by 2048**
- Malaysia's population reached **34.3 million in 2026**, while annual growth **slowed to 0.6%**
- Longer life expectancy and declining fertility rates are accelerating ageing trends

Urbanization & Demographic

- **Over 80% of Malaysians** now live in urban areas
- Rising urban affluence is increasing **demand for complex treatments**
- Active ageing, lifelong learning, and workforce participation are emerging priorities as **demographics shift**

Disease Burden

- Ischaemic heart disease remains the leading cause of death, accounting **for 15.1% of medically certified deaths**
- **NCDs continue to rise, with 30.9%** of adults classified as **obese** and **30.8% overweight**
- Pneumonia remains a significant health burden, particularly among elderly populations

Access & Regional Disparities

- Rural residents travel an average of **36km to hospitals versus 17km in urban areas**
- Healthcare access challenges remain significant **in Sabah and Sarawak,** particularly for specialist care
- **Geographic barriers** continue to limit digital health adoption and healthcare **connectivity in East Malaysia**

Sources: wargabiz, live population, scientific research; Spatial patterns of health clinic in Malaysia

Technological Outlook

Malaysia's healthcare sector is moving from fragmented systems toward integrated digital health records and cloud-based hospital platforms. This transition is supporting better patient data management and the implementation of DRG-based healthcare financing.

Core Drivers

1 **National Digital Health Platform**

MySejahtera is evolving beyond pandemic use cases into a national health portal, including integration of **90,000+ imaging records**.

2 **Health System Digitalization**

60% of public health facilities have migrated to the cloud, while THIS is expanding to **16 additional hospitals in 2026.**

3 **AI & Digital Healthcare**

National AI initiatives support early cancer detection, CT image analysis, and workforce productivity amid specialist shortages.

4 **Infrastructure & Interoperability**

Malaysia aims to digitalize all public hospitals **by 2028** while expanding digital systems **to 2,489 primary care facilities.**



Overview - How is technology reshaping healthcare industry?

- **2026 has been designated Malaysia's "Digital Transformation Year",** accelerating healthcare digitalization nationwide.
- **A national cloud-based clinic management system** is being rolled out to approximately **2,489 primary care facilities**.
- Technology is supporting healthcare financing reform through **DRG implementation, data integration, and value-based care initiatives**.

Sources: The Edge Malaysia, Maybank IB, hospitaltimes.co pic crd

Legal Outlook

Malaysia is introducing major healthcare reforms in 2026, including new digital health legislation, DRG-based payment reforms, and stronger oversight of healthcare pricing and service delivery.

Healthcare Laws & Compliance Framework

1 Private Healthcare Facilities & Services Act (PHFSA)

- Governs private hospitals, clinics, and healthcare service standards under Act 586
- Sets requirements for staffing, facility operations, and quality of care

2 Digital Healthcare Act (2026)

- Establishes legal oversight for telemedicine, AI, and electronic medical records
- Supports the national "One Individual, One Record" healthcare strategy

3 Medical Device Order 2026

- Brings aesthetic medical devices under formal MDA oversight
- Requires registration and use by trained healthcare personnel

Licensing & Operational Requirements

1 Provider Licensing

- Private facilities require Form 1 approval and Form 6 operating licences
- Licensing and inspections are administered by CKAPS under MOH

2 Professional Standards

- Doctors must maintain a valid Annual Practicing Certificate (APC)
- Specialists must be registered with the National Specialist Register (NSR)

3 Data & Privacy Compliance

- Healthcare providers must comply with PDPA requirements for patient information
- Non-compliance can result in penalties of up to RM500,000

Investment & Regulatory Reforms

1 Foreign Investment Framework

- 100% foreign ownership is permitted for private hospitals and specialty clinics
- MIDA offers a 100% Investment Tax Allowance on qualifying investments

2 Health White Paper Reforms

- Reforms support strategic purchasing and greater public-private collaboration
- Government increasingly purchases services from private providers

3 DRG & Pricing Reforms

- DRG implementation aims to improve pricing transparency and cost control
- Proposed amendments seek greater oversight of non-doctor hospital charges

Sources: MIDA Report, hbt law.com The Edge Malaysia

Environmental Outlook

Climate change, air pollution, and extreme weather events are increasingly influencing public health outcomes in Malaysia, while sustainability requirements are driving greater investment in green healthcare infrastructure and environmental compliance.



Climate & Environmental Risks

- **Respiratory-related outpatient** visits in the Klang Valley can rise by **20–30% during severe transboundary haze episodes**
- Climate change is projected to increase dengue cases by **approximately 15% in 2026** as temperatures and rainfall patterns shift
- Malaysia experiences recurring flood events**, with major floods in recent years** causing billions of ringgit in economic losses and infrastructure disruptions



Sustainability & Green Transition

- **SSB-aligned climate disclosures** are being adopted across listed companies, **strengthening ESG reporting requirements**
- The healthcare sector is implementing **RM21.3 billion of facility projects**, with **Green Hospital standards** increasingly incorporated into new developments
- **IHH Healthcare has targeted 50% renewable energy usage** across its Malaysian facilities, reflecting broader decarbonization efforts

Sources: pic crd Reuters, mgtc.gov.my, OCBC regional focus, New Strait Times

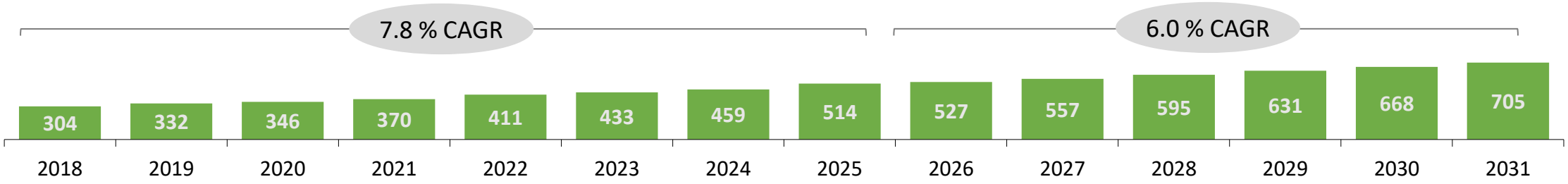
Macroeconomics

Vietnam's GDP is projected to grow by 6.2% in 2026 and 5.8% in 2027, supported by manufacturing expansion, export growth, and continued public and foreign investment.



Socio-economic Statistics	
GDP (2025)	USD 514 Bn
GDP / Capita (2025)	USD 5,026
GDP Growth (2026-2032)	6.0 % CAGR
Population (2025)	102.3 Mn
Urbanisation (2025)	38.49 %
Inflation rate (2025)	5.60 %

Vietnam GDP – Historical and Forecasts (USD Bn)



- Vietnam remains a key manufacturing hub, supported by **17 active FTAs**, continued supply chain diversification, **a workforce of ~68 million**, and productivity growth above **6.5% annually**.
- **Government initiatives** supporting industrial upgrading, renewable energy expansion, and financial market development are expected to **strengthen long-term economic growth and investment activity.**

Sources: International Monetary Fund (IMF), Statista, KPMG

Demographic & Culture

*Vietnam's 2026 market is a **rapidly growing, trade-oriented economy transitioning toward a consumption-led model fueled by an exploding middle class and heavy healthcare digitization.***

1 Golden Demographics

Vietnam is **capitalizing on a massive working-age base,** with an expanding, youthful population, driving a rapid **doubling of the country's middle class**.

2 High-Growth Economy

The economy maintains intense momentum **with a projected 6.8% GDP expansion** driven by public infrastructure and domestic manufacturing.

3 Emerging Consumers

Supported by **rising wages**, consumer habits are shifting structurally away from basic necessities toward **premium health and well-being products.**

4 Healthcare Expansion

Driven by the national budget**, public health spending is expanding rapidly** to improve primary care and build local private hospital capacity.

5 Pharma & Wellness

The pharmaceutical market is **scaling toward $14.6 billion** due to new policies favoring local production and rising chronic disease management.

6 Healthtech Acceleration

A strict government mandate requiring all clinics to implement **Electronic Health Records (EHR**) has triggered an unprecedented **local health IT boom**.

Sources: KPMG, World Bank, Vietcap, Fitch BMI, Trade.gov, Chambers

Total Household Spending

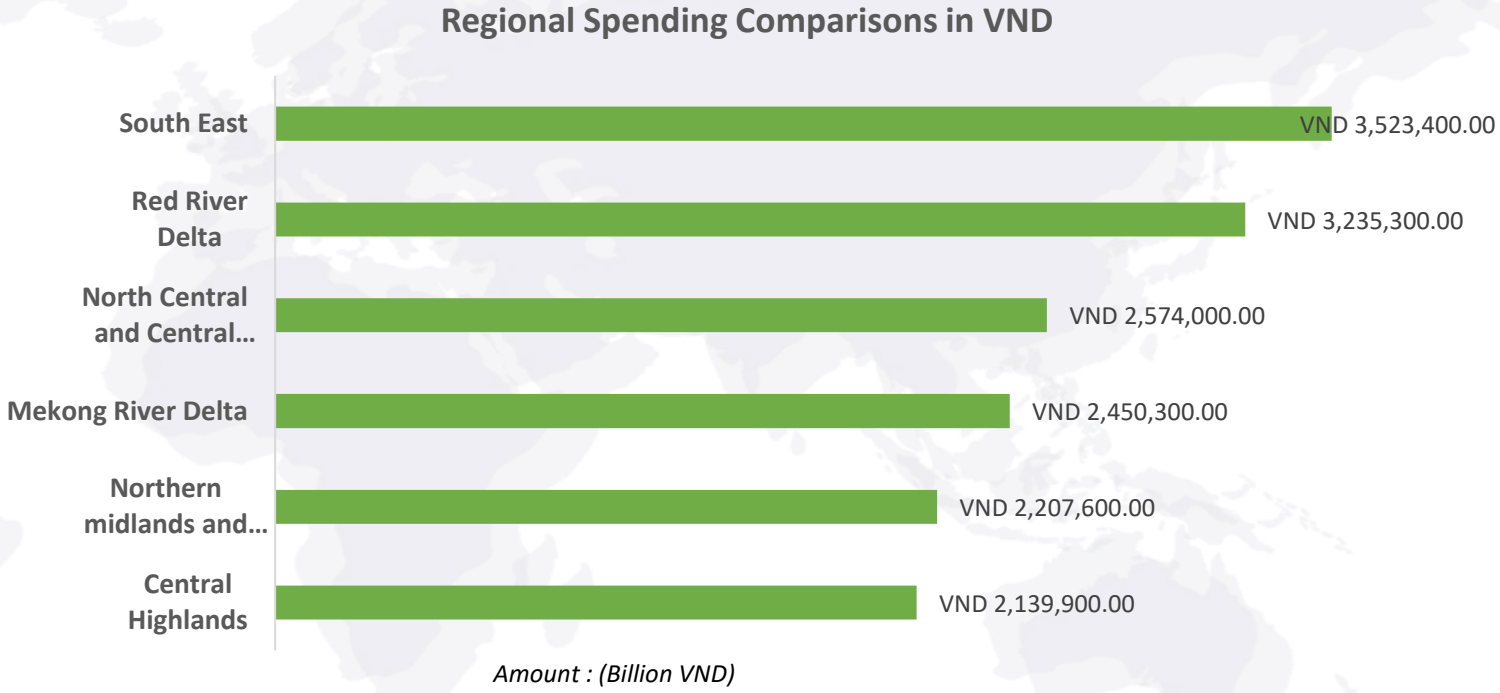
Vietnam's household spending is concentrated in the country's major economic regions, with the South East and Red River Delta benefiting from stronger income levels, urbanization, and consumer purchasing power.

National Monthly Household Expenditure

VND 2.8143 Mn

CAGR Growth of Household Expenditure

3.2 %

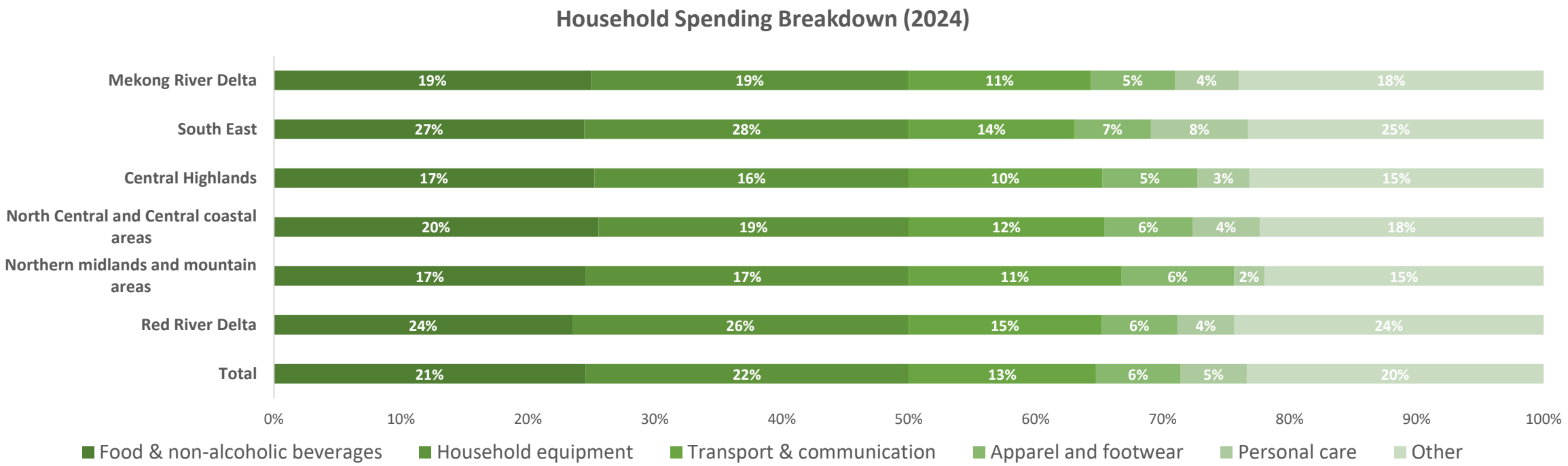


- **The South East and Red River Delta** lead national spending, with monthly expenditure exceeding VND 3.2 million, reflecting the concentration of economic activity around **Ho Chi Minh City and Hanoi.**
- The gap between leading urban regions and developing regions exceeds **VND 1.3 million per month,** highlighting **significant differences** in **purchasing power and consumer market maturity** across the country.

Sources: https://www.nso.gov.vn/

Total Household Spending Breakdown

Vietnam's spending profile remains centered on essential consumption, although more developed regions such as the South East and Red River Delta allocate a greater share of expenditure toward transport, communication, and discretionary purchases.



- **Food and household-related spending** account for **approximately 43% of total expenditure nationally**, highlighting the continued importance of essential consumption across households.
- **The South East allocates the highest share to transport and communication (14%) and personal care (8%),** reflecting higher incomes and urban consumer lifestyles.
- **Red River Delta and South East** households devote **nearly 75% of spending to core consumption categories**, indicating stronger purchasing power and demand for higher-value goods and services..

Sources: https://www.nso.gov.vn/

Healthcare Value Chain

Policy & regulation → Payers → Providers → Inputs → Access / customers

Value chain layer	Key players / components
Policy & regulation	MOH Vietnam, Vietnam Social Security, pharma/device regulators, digital-health authorities
Payers	BHYT, government budget, OOP, private insurance, employer benefits, self-pay private patients
Providers	Public hospitals; Vinmec, Hoan My, Tam Anh, FV, Thu Cuc, Hong Ngoc, Xuyen A, Hanh Phuc, MSG
Inputs	DHG, Traphaco, Imexpharm, Vinapharm, MNC pharma, medtech distributors, global devices
Access / customers	Public insured, urban middle class, specialty-care patients, pharmacy customers, digital users

Where commercial value is captured

- Private hospital chains capture patients escaping public overload and seeking speed/comfort.
- Specialty platforms such as eye care, maternity, fertility, dental, and diagnostics can scale repeatable models.
- Pharmacy retail captures recurring medicine and consumer-health demand.
- Medtech distributors capture import-led modernization demand from public/private hospitals.

Main value-chain bottlenecks

- Public hospitals are overloaded and quality varies by region/facility.
- Workforce shortages limit chain expansion and specialist services.
- Affordability constraints keep many patients price-sensitive.
- Regulatory complexity affects hospitals, pharma, devices, tenders, and foreign investment.

Sources: Sagasia internal research based on MOH digital transformation materials, VN health sector 2021–2025 update, FiinGroup Vietnam Private Healthcare Sector Preview, and U.S. ITA Vietnam Pharma Industry Updates.

Provider & Competitive Landscape

Market structure

Vietnam's provider market is moving into private chainization. Public hospitals still hold clinical authority, but branded private hospitals, specialty chains, diagnostics, and pharmacy networks are scaling in response to urban demand and public overcrowding.

Provider / group	Quantitative scale / hard numbers	Competitive positioning
Public hospitals	Central/provincial/district system; public insured care	Clinical authority, affordability, mass access
Private hospital market	400+ private hospitals by 2024; about 24% of all hospitals	Growing formal private provider base
Hoan My	2,800+ operating beds; 14 hospitals; 7 clinics	Scaled private hospital network
Vinmec	Premium national private healthcare system	Premium flagship, technology/research orientation
Tam Anh / FV	Major urban premium/private providers	Specialist, international-style, and high-trust private care
Medical Saigon Group	13 facilities: 8 eye hospitals and 5 general hospitals	Specialty-chain / ophthalmology platform

Strategic reading

Vietnam's competitive question is whether providers can turn isolated private hospitals into trusted, scalable systems. The winning model combines clinical credibility, quality standardization, urban demand capture, and specialty-chain economics.

Sources: Sagasia internal research based on FiinGroup Vietnam Private Healthcare Sector Preview, B&Company private hospital chainization analysis, Hoan My company materials, and Reuters reporting on Medical Saigon Group and Pinnacle Health Equipment transactions.

Customer Segments & Demand-Side View

Demand-side context

Vietnam's demand side is driven by broad insurance coverage, overloaded public hospitals, and rising urban willingness to pay. The private opportunity is not mature medical tourism; it is domestic modernization: faster access, better service, trusted specialists, diagnostics, pharmacy retail, and branded chain care.

Customer segment	Core demand / need	Likely provider choice
Public insured patients	Affordable treatment, public insurance, hospital access	Public hospitals, provincial/district hospitals
Urban middle class	Faster service, cleaner facilities, diagnostics, branded care	Vinmec, Hoan My, Tam Anh, FV, Thu Cuc, Hong Ngoc
Premium private patients	Trust, specialists, privacy, advanced equipment	Vinmec, FV, Tam Anh
Maternity / family patients	Obstetrics, pediatrics, fertility, women's health	Hanh Phuc, Tam Anh, private hospitals
Specialty-care patients	Eye care, dental, oncology, cardiology, diagnostics	Medical Saigon Group, specialty clinics, private hospitals
Pharmacy / chronic users	Medicines, refills, OTC, consumer health	Long Chau, Pharmacity, An Khang, local pharmacies

Quantitative demand signals

- Health insurance coverage: 95.15%; 15 doctors and 34.5 beds per 10,000 people.
- Private healthcare expenditure expected at 7.5% CAGR from 2025–2030.
- Major urban public hospitals can operate up to 200% of intended capacity.

Provider implications

- Private providers can win by solving speed, comfort, and trust gaps.
- Specialty chains should focus on repeatable, high-demand categories.
- Digital patient experience and pharmacy integration can differentiate providers.

Sources: Sagasia internal research based on Vietnam News health sector 2021–2025 update, FiinGroup Vietnam Private Healthcare Sector Preview, VietnamPlus health insurance reporting, and WHO Vietnam public health review.

Brand Positioning Axis



Entry barriers

- Major public hospitals retain strong clinical trust, especially for complex care.
- Private providers must prove outcomes, doctor quality, safety, and pricing fairness.
- Workforce shortages and quality standardization limit scaling.
- Affordability remains a constraint even as middle-class demand rises.

Provider moats

- Vinmec: premium brand, Vingroup backing, technology and quality positioning.
- Hoan My: network scale, operating experience, middle-market reach.
- Tam Anh/FV: urban premium specialist and international-standard positioning.
- Medical Saigon Group: focused specialty-chain model and ophthalmology base.

Success factors

- Build patient trust, not only comfort or aesthetics.
- Standardize clinical quality and service across sites.
- Target Hanoi/HCMC urban middle class while expanding selectively.
- Develop specialty platforms: eye, fertility, maternity, diagnostics, cardiology, oncology.

Strategic reading

Vietnam is in chainization mode. Winners are providers that can build brand trust, standardize quality and scale private/specialty models beyond isolated urban hospitals.

Sources: Sagasia internal research based on FiinGroup Vietnam Private Healthcare Sector Preview, B&Company private hospital chainization analysis, Reuters reporting on Medical Saigon Group, Asian Scientist Vinmec feature, and U.S. ITA Vietnam Pharma Industry Updates.

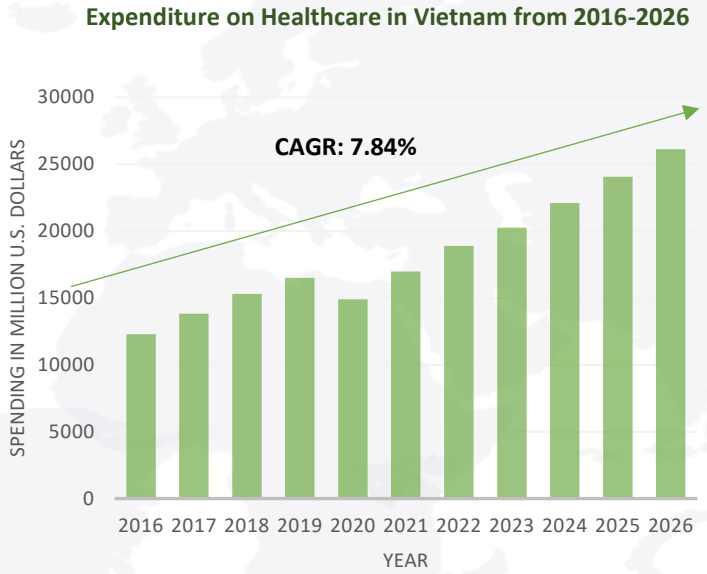
Political Outlook

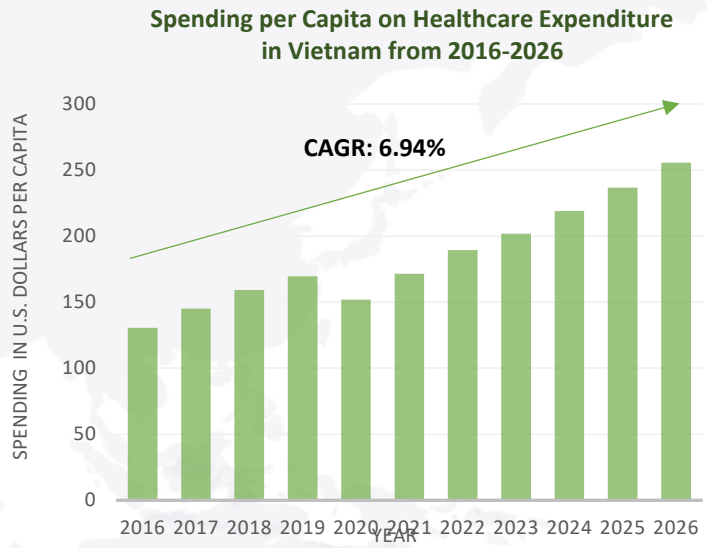
Vietnam's highly stable political system continues to support long-term policy continuity and healthcare investment. Recent reforms are focused on expanding healthcare access, strengthening primary care, and accelerating nationwide digital integration.

Universal Health Coverage

96 % Insurance Coverage by 2026

Development Priorities	• Vietnam aims to become **an upper-income economy by 2030** and a **high-income economy by 2045** • The government has proposed **a US$4.75bn healthcare investment program for 2026–2035,** focused on primary and preventive care
Healthcare Modernization	• All citizens will receive **at least one free annual health screening from 2026**, shifting care toward prevention and early detection • **Electronic health records** will be integrated into the national **VNeID platform**, enabling lifelong digital health management





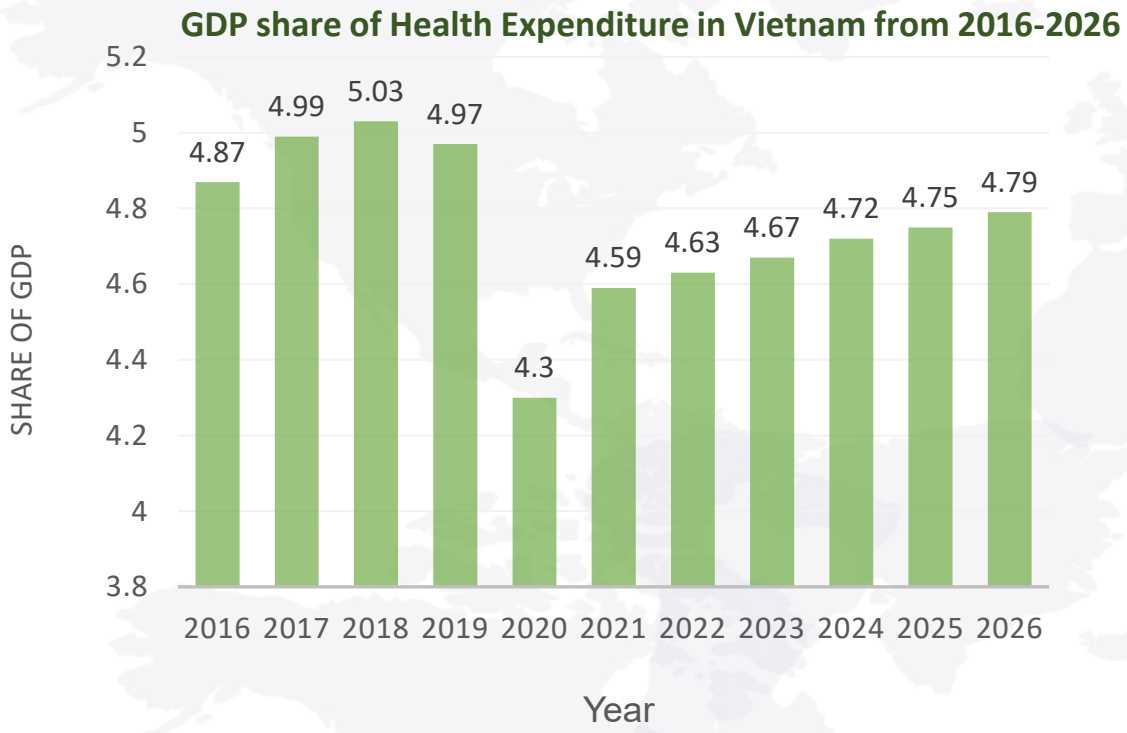
Policy Environment

- **Phase 1 of healthcare investment (2026–2030)** allocates **VND43tn for grassroots healthcare** and **VND18.7tn for preventive medicine expansion**
- **Resolution 72 supports healthcare workforce development**.

Sources: WHO, Statista, exportfinance.gov.au, virtnamplus

Economic Outlook

Vietnam's healthcare market remains in an expansion phase, driven by rapid economic growth and rising healthcare utilization. New affordability reforms are expected to gradually reduce the sector's historically high reliance on out-of-pocket spending



- **Vietnam's GDP** is projected to **grow 6.0–7.8% in 2026**, supported by strong domestic demand and continued economic development
- **Healthcare spending** is growing **at a 7.6–9.7% CAGR**, with per capita expenditure expected to reach **US$706 (PPP) by 2026**
- Public health insurance coverage **has expanded to 93–95%** of the population, supporting broader healthcare utilization
- **Total healthcare expenditure** is projected to reach **US$33.8bn by 2030** as healthcare becomes a higher budgetary priority

Key Insights

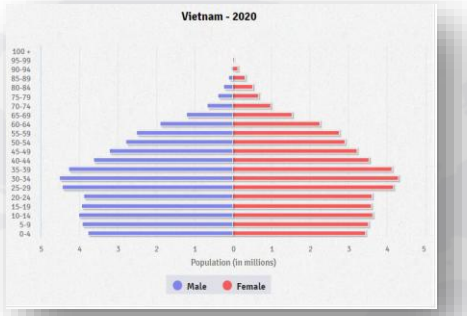
- Out-of-pocket spending remains high at 40–46% of total healthcare expenditure, highlighting the continued financial burden faced by many households despite expanding insurance coverage
- A new hospital fee exemption policy will gradually shift VND21.5tn of annual medical costs from households to the state budget, beginning with free annual checkups for all.
- Vietnam remains a cost-competitive healthcare destination, with treatment costs estimated to be 30–50% lower than Singapore and Thailand, supporting both domestic affordability and medical tourism potential .

Sources: Commonwealth Fund, KPMG, OECD, Statista, Fitch Solution, Report Linker

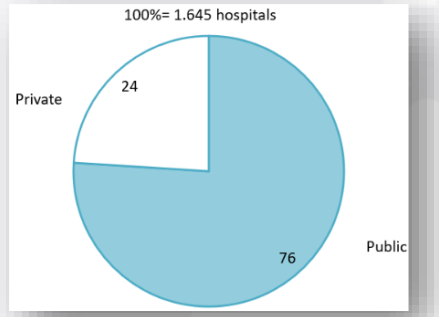
Social Outlook: Demographics, Disease Burden & Healthcare Access

Vietnam is undergoing one of the world's fastest demographic transitions, moving from a young population to an aged society within a single generation. Rapid ageing, rising chronic disease prevalence, and healthcare disparities are reshaping long-term healthcare demand.









Aging Population

- Vietnam became **an ageing society in 2015** and is projected to become **an aged society by 2035–2036**
- The population **aged 65+ is expected to double by 2035**, making it **one of Asia's fastest-ageing countries**
- People aged 60+ are projected to exceed 25% by 2050

Demographic Shift

- Vietnam's **working-age population has been declining** since 2014, gradually reducing its demographic dividend
- Urbanization has reached **40–42%**, driving migration toward major cities
- Vietnam's demographic window of **opportunity** is expected to **close around 2040–2042** as ageing accelerates

Disease Burden

- **NCDs** dominate the healthcare system, with disease burden **rising from 51% in 1990 to 74% in 2017**
- Older adults account for **roughly 24% of total DALYs** lost, reflecting growing chronic disease pressures
- **Disability rates rise sharply with age**, affecting over 40% of people aged 65 and above

Access & Inequality

- **Hospital overcrowding remains a major challenge**, with occupancy rates at major urban hospitals often **exceeding 150%**
- **Healthcare access gaps** persist **between urban and rural regions**
- **Older populations** are increasingly **concentrated in rural provinces**, creating **additional pressure on local healthcare services**

Sources: WHO, Index Mundi, Vir.com, MOH

Technological Outlook

With occupancy rates at major central hospitals frequently exceeding 120%, Vietnam is increasingly using telehealth and digital technologies to expand specialist care beyond overcrowded urban facilities.

Core Drivers

1 Digital Health Integration

Approximately 90% of provincial hospitals have adopted **electronic health records,** supporting nationwide digital health initiatives.

2 Telemedicine & Remote Care

Vietnam's Telehealth platform links 1,500 local clinics with specialist hospitals, expanding access to expert care in underserved areas.

3 AI & Smart Diagnostics

AI-assisted solutions are increasingly used for radiology, oncology triage, and tuberculosis detection to support healthcare capacity.

4 Healthcare Modernization

Vietnam is **the fastest-growing ASEAN market for advanced diagnostic equipment**, supported by upgrades to provincial diagnostic centers and private hospital expansion.



Overview - How is technology reshaping healthcare industry?

- **Over 50 million citizens** can now access **health records through the national VNeID digital identification platform**
- **The national Telehealth network** connects **approximately 1,500 grassroots clinics** with leading hospitals across the country
- AI adoption is accelerating in radiology, oncology, and tuberculosis screening to support earlier diagnosis and clinical efficiency

Sources: Qubika pic crd, Fitch Solutions, KPMG Vietnam, Freshfields

Legal Outlook

Vietnam is raising healthcare quality standards through new national licensing requirements while maintaining one of Southeast Asia's most attractive regulatory environments for healthcare investment.

Healthcare Laws & Compliance Framework

1 Law on Medical Examination & Treatment (2023)

- Fully implemented in 2026, introducing a service-capability-based licensing model
- Shifts regulatory focus toward healthcare quality and clinical competency

2 Healthcare Pricing & Reimbursement

- Public hospital pricing is standardized under MOH regulations
- Social Health Insurance (SHI) covers approximately 80–100% of eligible treatment costs

3 Preventive Healthcare Reform

- Resolution 72 introduces free annual health screenings for citizens
- Supports a national shift toward preventive and primary healthcare

Licensing & Professional Requirements

1 Facility Licensing

- Hospitals are licensed based on clinical capability, staffing, and infrastructure standards
- Requirements include specialist staffing ratios and environmental compliance

2 National Competency Assessment

- Clinicians must pass a national competency examination to obtain practice licenses
- Represents one of Vietnam's most significant healthcare quality reforms

3 Continuing Medical Education (CME)

- Practice licenses are valid for five years under the new framework
- Renewal requires completion of 120 hours of continuing education

Investment & Market Access

1 Foreign Investment Framework

- 100% foreign ownership is permitted in healthcare facilities
- Eligible projects may receive a preferential 10% CIT rate versus the standard 20%

2 Healthcare Investment Incentives

- Healthcare remains a specially encouraged investment sector under Vietnamese law
- Qualified projects may receive land-rent exemptions and investment incentives

3 Public-Private Partnerships (PPP)

- The government encourages PPP participation to reduce pressure on major hospitals
- Private investment is increasingly used to expand healthcare infrastructure

*Sources: **MOH**, **National Assembly Portal**, **Vietnam Law Magazine**, and **VnExpress**.*

Environmental Outlook

Vietnam's rapid urbanization and high climate vulnerability are intensifying environmental risks, while the country's Net Zero 2050 commitment is accelerating sustainability and environmental compliance efforts.



Sustainability & Environmental Transition

- **Hanoi** frequently records **PM2.5 concentrations several times above WHO air quality guidelines**, contributing to growing respiratory and cardiovascular health concerns
- Vietnam is **ranked among the countries most vulnerable to climate change**, with the **Mekong Delta facing rising risks** from flooding, saltwater intrusion, and sea-level rise
- **Rising temperatures** and **changing rainfall patterns** continue to increase dengue transmission risks and climate-sensitive disease across urban centers



Sustainability & Environmental Transition

- Vietnam has **committed to achieving Net Zero emissions by 2050**, driving greater investment in renewable energy and sustainable infrastructure
- **Updated healthcare waste management regulations** are encouraging hospitals to adopt cleaner treatment technologies and strengthen waste segregation practices
- **Green building and energy-efficiency standards are increasingly incorporated** into new public infrastructure projects, supported by international climate financing initiatives

Sources: pic crd east asia forum, special.vietnamplus, IQAir, UNDP,Vietnam News.

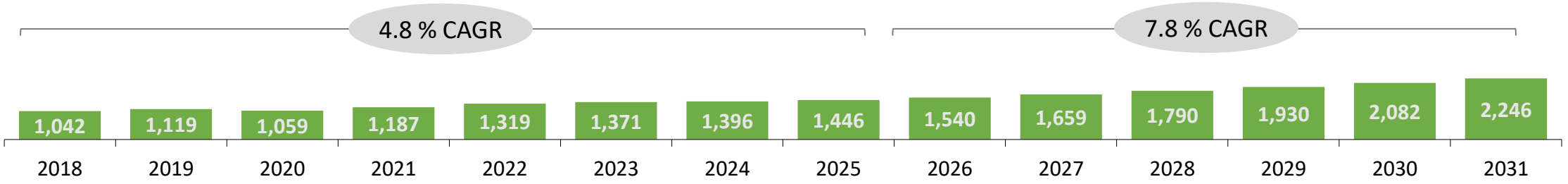
Macroeconomics

Indonesia's economy is expected to maintain around 5% growth over the medium term, supported by domestic consumption, infrastructure investment, and government-led industrial development, while 1Q26 GDP grew 5.61%, the fastest in over 3 years.



Socio-economic Statistics	
GDP (2025)	USD 1,446 Bn
GDP / Capita (2025)	USD 5,083
GDP Growth (2026-2032)	7.8 % CAGR
Population (2025)	287.2 Mn
Urbanisation (2025)	59.6 %
Inflation rate (2025)	2.92 %

Indonesia GDP – Historical and Forecasts (USD Bn)



- Public investment is expected to accelerate through 2026–2027, supported by a sovereign wealth fund and government spending, while fiscal deficits remain below the statutory limit of **3.0% of GDP**.
- Public debt is projected to remain around 40% of GDP, supporting continued investment in **infrastructure, clean energy, health, and education**.

Sources: International Monetary Fund (IMF), Statista, OECD

Demographic & Culture



Indonesia's 2026 market is a vast consumer powerhouse driven by solid economic growth, a highly digitizing populace, and critical structural transformations in universal health coverage.

1 Gotong Royong Culture

Local culture prioritizes "**Gotong Royong**" **(mutual aid),** driving near-universal public participation and alignment with national health insurance initiatives.

2 Resilient Economy

The massive domestic market sustains strong economic momentum, backed by **a stable 4.9%–5.1% GDP expansion** forecast.

3 Telehealth Adoption

Massive consumer adoption of platforms like Halodoc makes decentralized, app-based doctor consultations a daily routine for urban families.

4 Universal Health Net

The national health insurance scheme (JKN/BPJS) has scaled rapidly, bringing baseline medical coverage to 96% of the population.

5 Specialist Deficit

The market faces critical national gaps in specialist doctors, driving heavy provider interest in foreign medical talent and automation.

6 SATUSEHAT Integration

New government mandates **require over 60,000 healthcare clinics** to unify completely into the centralized national data network..

Sources: Emerald Insight, Bank Indonesia, Research and Markets, FutureCFO, WHO, Standard Chartered

Total Household Spending

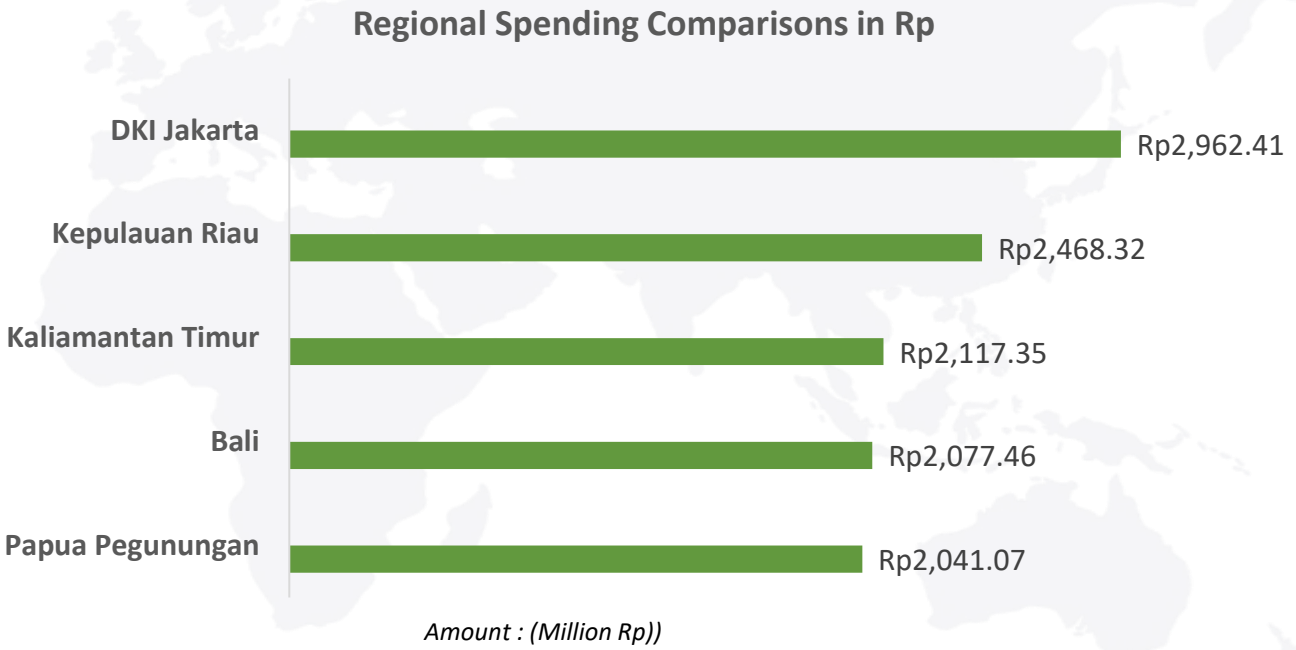
Indonesia's consumer spending is concentrated in economically strategic regions, with Jakarta maintaining the strongest purchasing power while resource-rich and tourism-driven provinces also demonstrate significant consumer market potential.

National Monthly Household Expenditure

IDR 6.1 Mn

CAGR Growth of Household Expenditure

3.8 %

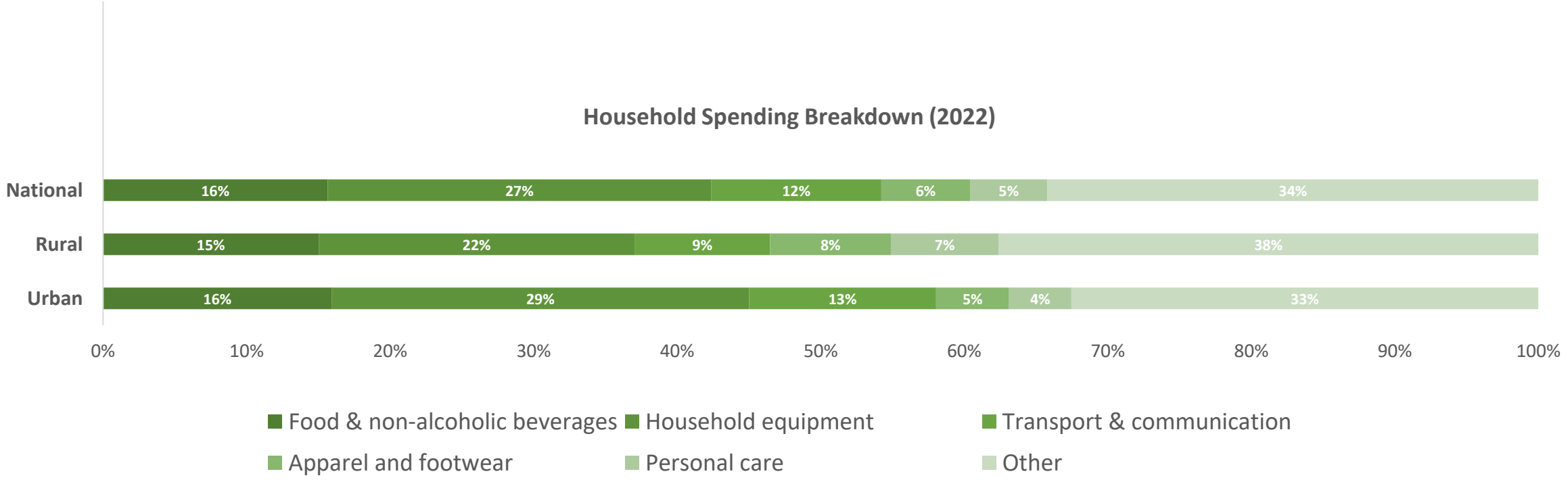


- **DKI Jakarta l**eads household spending at nearly **Rp3.0 million per month**, reinforcing its position as Indonesia's primary consumer and economic hub.
- **Kepulauan Riau, East Kalimantan, and Bali** rank among the country's **highest-spending regions**, reflecting the influence of industrial activity, natural resources, and tourism on household purchasing power.

Sources: Statista, BPS-Statistics Indonesia Portal.

Total Household Spending Breakdown

Indonesia's spending profile remains heavily focused on household necessities, although urban households allocate a greater share of expenditure toward transportation, communication, and lifestyle-oriented consumption.



- **Household-related expenditure** accounts **for the largest share of spending at 27% nationally**, rising to 29% in urban areas, reflecting the higher living cost in cities.
- **Urban households** spend a larger proportion on transport and communication **(13% vs. 9% in rural areas**), highlighting greater mobility and digital connectivity.
- Food and household-related categories together account for approximately **43% of national expenditure**, underscoring the continued importance of **essential consumption in household budgets**

Sources: BPS-Statistics Indonesia Portal.

Healthcare Value Chain

Policy & regulation → Payers → Providers → Inputs → Access / customers

Value chain layer	Key players / components
Policy & regulation	Kemenkes, BPOM, regional governments, hospital regulators
Payers	BPJS/JKN, government budget, private insurance, corporate benefits, OOP
Providers	Public hospitals, puskesmas; Siloam, Hermina, Mitra Keluarga, Mayapada, EMC, Primaya
Inputs	Bio Farma, Kalbe, Dexa, Kimia Farma, global medtech, distributors, SATUSEHAT
Access / customers	JKN patients, urban middle class, premium domestic patients, chronic disease patients

Where commercial value is captured

- Private hospital chains capture middle-class, corporate, non-JKN, and selected JKN demand.
- Premium providers can retain Indonesians who would otherwise travel to Singapore/Malaysia.
- Diagnostics and medtech distribution benefit from hospital expansion and NCD screening.
- Digital health can solve claims, referrals, telemedicine, and geographic access pain points.

Main value-chain bottlenecks

- Archipelagic geography complicates logistics, referrals, maintenance, and specialist access.
- Workforce shortages and uneven doctor/specialist distribution constrain growth.
- BPJS reimbursement pressure can compress margins.
- Quality variation creates patient distrust and outbound leakage.

Sources: Sagasia internal research based on Indonesia MOH Health Transformation materials, MOH Strategic Plan 2025–2029, World Bank National Health Compact Indonesia, and WHO Indonesia Medical Products Profile 2025.

Provider & Competitive Landscape

Market structure

Indonesia is not a mature medical-tourism market; it is a domestic access and capacity market. Public providers carry mass JKN demand, while private chains scale across cities to capture middle-class, corporate, and premium care pockets.

Provider / group	Quantitative scale / hard numbers	Competitive positioning
Public hospitals / puskesmas	National public network tied to BPJS/JKN	Mass access, primary care, referrals, public legitimacy
Siloam Hospitals	41+ hospitals and 25 clinics	Scaled private hospital platform; upper-mid/private-pay push
Hermina	51 hospitals, 36 cities, 8,287 beds	Mass family hospital chain; broad domestic and BPJS relevance
Mitra Keluarga	1,400 pipeline beds; 2,257 potential additions	Urban private-pay / affluent catchment player
Mayapada	7 hospitals; 1,000+ beds; target 2,000+ by 2027	Premium challenger with PE-backed expansion
EMC / Primaya / Awal Bros	Regional/private chain presence	Middle-class and regional private care

Strategic reading

Indonesia’s provider strategy depends on payer mix and execution. The winning model combines scale, region-by-region discipline, workforce access, BPJS claims capability, and a private-pay offering strong enough to avoid margin compression.

Sources: Sagasia internal research based on Siloam Hospitals investor information, Mitra Keluarga 1H25 company presentation, Bain Capital Mayapada Healthcare investment announcement, Hermina 2025 results coverage, and Reuters healthcare transaction reporting.

Customer Segments & Demand-Side View

Demand-side context

Indonesia's demand-side story is massive but domestic. JKN/BPJS has formalized access for most of the population, but capacity, workforce, quality, and regional access lag demand. The private opportunity is in solving access and quality gaps while managing BPJS economics.

Customer segment	Core demand / need	Likely provider choice
JKN/BPJS mass patients	Affordable basic and hospital care through public insurance	Puskesmas, public hospitals, BPJS-contracted private hospitals
Low-income subsidized patients	Local access, medicine availability, minimal direct payment	Puskesmas and public hospitals
Urban middle class	Shorter waits, better facilities, diagnostics, specialists	Siloam, Hermina, Mitra Keluarga, Primaya, EMC
Premium domestic patients	Complex care, privacy, doctor trust, advanced equipment	Mayapada, Siloam flagship, Mitra Keluarga
Outbound-risk patients	Singapore/Malaysia-level confidence for oncology/cardiac/complex care	Premium domestic hospitals or overseas hospitals
Chronic disease patients	Long-term medicine, monitoring, dialysis, diabetes/cardiac care	Public/private hospitals, clinics, pharmacies

Quantitative demand signals

- JKN coverage: 97.6%; private insurance: 0.8%; OOP share: ~31%.
- Free screening target: 100m people in 2025 through 20,000+ centers/clinics.
- Specialist bottleneck remains acute: limited specialist availability vs population scale.

Provider implications

- Providers must balance BPJS volume with private-pay/corporate segments.
- Premium hospitals need trust-building to reduce outbound leakage.
- Diagnostics, chronic disease care, and digital access tools should see strong demand pull.

Sources: Sagasia internal research based on Commonwealth Fund Indonesia Health System Profile, Reuters reporting on Indonesia's national health screening program, and World Bank National Health Compact Indonesia.

Brand Positioning Axis



Entry barriers

- Geography makes multi-region scaling, logistics, referral pathways, and device maintenance difficult.
- BPJS/JKN rules require claims, tariff, and margin-management capabilities.
- Workforce shortages are severe and uneven across regions.
- Hospital capex and licensing require long payback and local execution discipline.

Provider moats

- Siloam: national scale, brand, hospital network, high-intensity/private-pay direction.
- Hermina: broad family-care reach, domestic trust, JKN familiarity.
- Mitra Keluarga: affluent urban catchments and quality perception.
- Mayapada: premium positioning, capital backing, centers-of-excellence ambition.

Success factors

- Manage payer mix: do not become trapped in low-margin BPJS volume.
- Scale regionally without losing clinical and service standardization.
- Develop specialist lines: oncology, cardiac, dialysis, maternity, neurology, diagnostics.
- Use digital systems for claims, referrals, patient records, and telehealth reach.

Strategic reading

Indonesia is a capacity buildout market. Provider competition is shaped by BPJS exposure, private-pay capture, geographic execution and ability to standardize quality across regions.

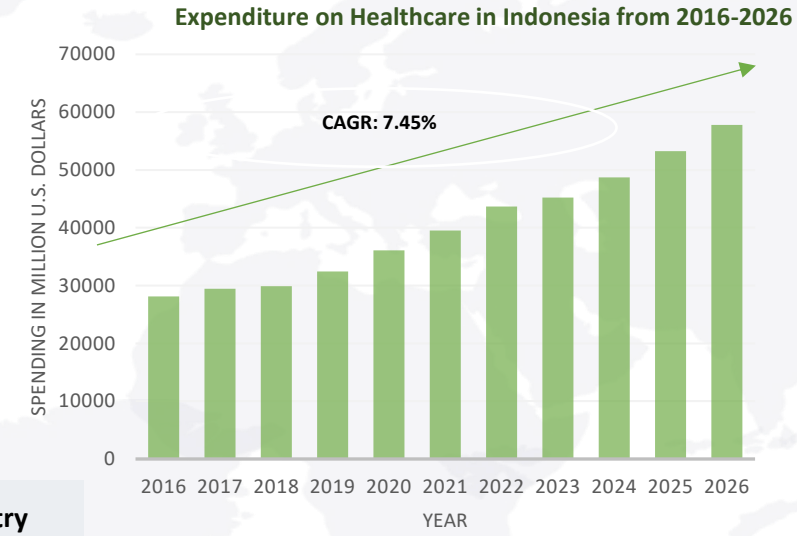
Sources: Sagasia internal research based on Siloam Hospitals investor information, Mitra Keluarga 1H25 company presentation, Bain Capital Mayapada Healthcare investment announcement, Commonwealth Fund Indonesia Health System Profile, and Indonesia MOH Strategic Plan 2025–2029.

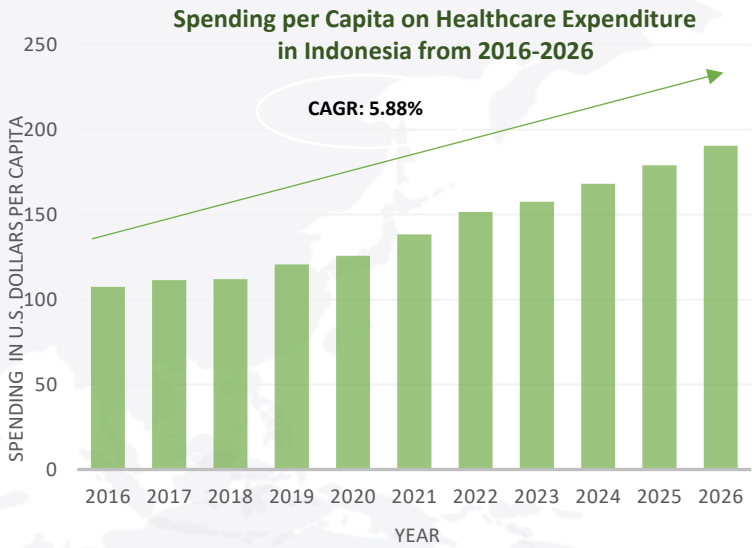
Political Outlook

The Prabowo administration has emphasized policy continuity and public investment as key pillars of national development. Healthcare, infrastructure, and human capital remain central priorities under the government's long-term reform agenda.

JKN National Health Insurance

98 % Population Coverage





National Priorities

- **Indonesia Emas 2045** targets **developed-country status** through investments in healthcare, education, infrastructure, and human capital
- **Reducing development gaps** across more than **17,000 island**s remains a national priority

Healthcare Capacity Development

- **The RIBK 2025–2029** roadmap focuses on improving healthcare quality, infrastructure capacity, and service accessibility across Indonesia
- Government initiatives continue **to prioritize healthcare workforce development** and reducing regional disparities in healthcare access

Policy Environment

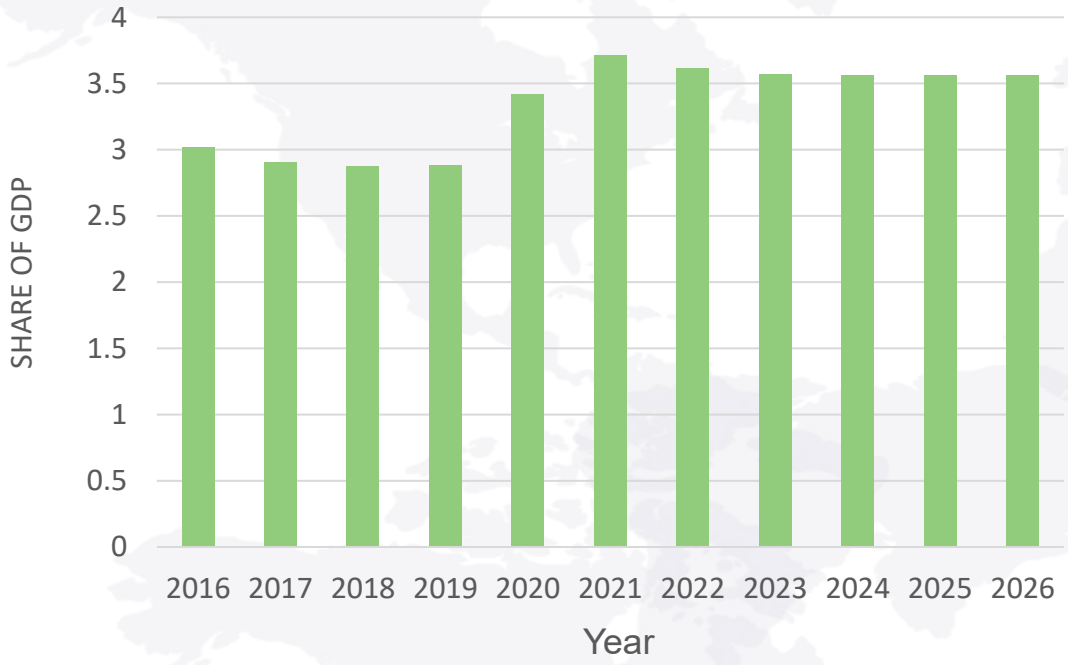
- **The 2026 healthcare budget** was increased to **Rp228tn (US$13.8bn)**, supporting **JKN** expansion and healthcare system strengthening
- **JKN** now covers approximately **98% of the population**, making it one of the world's largest national health insurance programs

Sources: setkab.go.id, World Bank, Statista, Fitch Ratings

Economic Outlook

Indonesia combines near-universal healthcare coverage with one of the lowest healthcare spending ratios among major ASEAN economies. This creates opportunities for future investment as the government seeks to improve healthcare access and infrastructure.

GDP share of Health Expenditure in Vietnam from 2016-2026



- **Indonesia's GDP** is projected to grow **5.5–5.8% in 2026**, supported by domestic consumption, infrastructure investment, and public spending
- Healthcare spending reached **Rp639.9tn in 2024** and has nearly doubled over the past decade, driven by utilization **under JKN**
- **JKN coverage** has expanded to **approximately 98–99%** of the population, making it one of the world's largest national health insurance systems
- Healthcare expenditure remains modest at **approximately 2.9–3.1% of GDP,** suggesting substantial long-term growth potential as healthcare demand rises

Key Insights

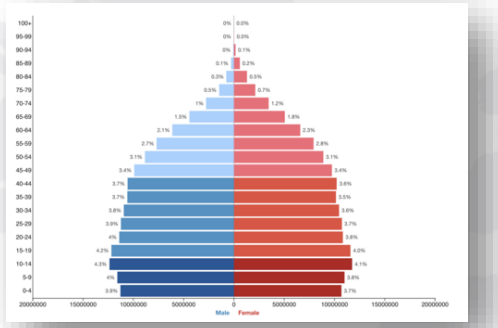
- Out-of-pocket spending declined to 28.3% of total health expenditure in 2024, reflecting improved financial protection through JKN expansion, though household cost burdens remain significant.
- Public financing now accounts for 58.5% of total healthcare expenditure, reinforcing the government's central role in healthcare funding and service delivery
- Despite near-universal coverage, healthcare access and service quality remain uneven across Indonesia's 17,000+ islands, supporting continued investment in healthcare.

Sources: Statista, ANTARA News, InvestorTrust

Social Outlook: Demographics, Disease Burden & Healthcare Access

Indonesia is experiencing rapid urbanization and demographic ageing, increasing demand for healthcare services across the archipelago. At the same time, rising NCD prevalence and persistent geographic disparities continue to challenge healthcare access and delivery.









Aging Population

- Indonesia's population **aged 60+ is projected to exceed 19%** by 2045, nearly doubling from current levels
- **Life expectancy continues to improve**, contributing to growing demand for chronic and elderly care
- **Population ageing is accelerating**, requiring greater healthcare and social support infrastructure

Demographic Shift

- **More than 58% of Indonesians** now live in urban areas, with continued migration toward major cities
- Indonesia's population **exceeds 280 million**, creating significant demand for healthcare capacity
- Government programs aim to improve child health outcomes, including **free meal initiatives to 82.9 million beneficiaries**

Disease Burden

- **NCDs** account for the majority of deaths, with **cardiovascular disease, diabetes, and cancer becoming prevalent**
- **Hypertension, obesity, and diabetes** continue to rise alongside urbanization and lifestyle changes

Access & Inequality

- Healthcare **access remains uneven across Indonesia's 17,000+ islands** despite near-universal insurance coverage
- **Rural and remote regions** continue to face **shortages of healthcare professionals** and specialist services
- Programs such as **Nusantara Sehat** and digital health initiatives aim to **strengthen primary care access**

Sources:Tempo.co, MOH, Jakarta Post

Technological Outlook

Indonesia is accelerating healthcare digitalization through SATUSEHAT and nationwide EMR adoption, helping connect healthcare services across more than 17,000 islands and a population exceeding 280 million.

Core Drivers

1 National Health Data Platform

SATUSEHAT has successfully integrated over **34,400 healthcare facilities**, including **91% of primary care centers and 95% of hospitals**, into a single national health ecosystem**.**

2 Digital Health Transformation

EMR implementation has achieved **over 96% compliance** across hospitals, standardizing **global HL7 FHIR protocols** to create seamless medical data accessibility

3 Telehealth & Remote Access

Digital healthcare utilization has exploded by **over 440% post-pandemic, e**nabling virtual doctor access and decentralized remote care

4 Infrastructure & Capacity

The broader digital transformation market is scaling **toward $29.03 billion**, backed by **massive cloud infrastructures** to process advanced AI diagnostic capacity. .



Overview - How is technology reshaping healthcare industry?

- **Centralized cloud platforms** successfully consolidated **over 400 isolated provincial legacy application**s into a secure, **unified sovereign database ecosystem.**
- Deep-learning algorithms **deployed across 38 provinces** dramatically cut diagnostic backlogs for regional high-burden cases like tuberculosis and stroke.
- Unified data protocols helped **eliminate a massive IDR 7.5 trillion public deficit,** shortening hospital insurance reimbursement cycles.

Sources: alberta innovates pic crd, MOH, HealthTech Asia, and Black Book Research

Legal Outlook

Indonesia's healthcare regulatory framework has been fundamentally reshaped by Health Law No. 17 of 2023, which liberalizes investment, streamlines licensing, and accelerates healthcare digitalization through the SATUSEHAT ecosystem.

Healthcare Laws & Compliance Framework

1 Health Law No. 17 of 2023

- Consolidates multiple healthcare laws into a single regulatory framework
- Strengthens MOH authority and simplifies healthcare governance

2 Digital Health Requirements

- Healthcare facilities are required to integrate with SATUSEHAT
- Mandatory EMR implementation supports national interoperability

Licensing & Professional Requirements

1 Facility Licensing

- Healthcare facilities are licensed through the OSS system
- Hospitals are classified based on service capability and complexity

2 Medical Professional Licensing

- Lifetime STR registration introduced under the Omnibus Health Law
- Practice licences (SIP) remain subject to competency-based renewal

3 Foreign Medical Professionals

- Licensing requirements for foreign specialists have been simplified
- Reforms aim to address shortages in specialist healthcare services

Investment & Market Access

1 Foreign Investment Framework

- Foreign participation in hospitals has been significantly liberalized
- Special Economic Zones provide additional incentives for healthcare projects

2 Healthcare Investment Incentives

- Sanur Health SEZ offers tax incentives and streamlined approvals
- Policies support healthcare infrastructure and medical tourism development

Sources: ABNR Law, Hiswara Bunjamin & Tandjung (HBT), and International Trade Administration

Environmental Outlook

Environmental challenges are becoming increasingly significant in Indonesia, with worsening air quality and climate-related disasters driving sustainability initiatives and long-term resilience planning.



Climate & Environmental Risks

- **Jakarta** regularly ranks among **the world's most polluted major cities**, with PM2.5 concentrations frequently exceeding WHO air quality guidelines by several multiples
- Indonesia is **highly vulnerable to flooding**, extreme rainfall, and sea-level rise, with coastal cities such as **Jakarta facing increasing climate adaptation challenges**
- Climate change and environmental degradation continue **to heighten risks from vector-borne diseases, heat stress, and disaster-related health impacts**



Sustainability & Environmental Transition

- **Indonesia** has committed to achieving Net Zero emissions **by 2060 or earlier**, accelerating investments in renewable energy and sustainable infrastructure
- **The government is promoting a Sustainable Health System framework** that prioritizes preventive care, digital health integration, and resource-efficient service delivery
- Healthcare providers and public institutions **face increasing expectations to improve waste management, reduce emissions, and strengthen environmental sustainability practices**

Sources: pic crd thejakartapost, greenwise, IQAir, The World Bank, WHO, and MOH.